

PROFESSIONAL NURSING COACH

MIDWIFERY / OBG NURSING II

PNC ULTRA-LMR HYBRID MASTER COMPENDIUM • NUR-20 • VERIFIED

60 Theory + 40 Skill Lab + 320 Clinical hours • 5 Units • Batches 1–9 • 110 LMR pages + cover & TOC • NORCET / RRB / ESIC / DSSSB

Batch	Units	Pp
B1	I-A: risk, hyperemesis, abortions, ectopic, mole, APH, anemia, HTN	13
B2	I-B: GDM, comorbidities, multiples, FGR, groups, drugs	15
B3	II-A: preterm, PROM, OP, breech, face, pelvis, labour patterns	13
B4	II-B: retained, injuries, PPH, rescue, distress, rupture, prolapse	12
B5	II-C: dystocia, inversion, shock, epi, instruments, CS, drugs	8
B6	III: sepsis, sec PPH, hematoma, breast, DVT, mood, fistula	12
B7	IV: levels, preterm, KMC, asphyxia, sepsis, RD, jaundice, feeds	14
B8	V-A: gynae, AUB, PID, prolapse, endo, fibroid, cancers, HRT	14
B9	V-B: infertility, ART, surrogacy, violence, procedures, flags	9

WhatsApp DM: +91 90571 22020 • Hybrid: navy hero + chips + teal bars + Trap/Hit + Pearl + Speed-check • v1.0 Sep-2026

Pp	Block	Focus
4-16	I-A (B1)	Risk, hyperemesis, abortions, ectopic, mole, APH, previa, abruptio, anemia, HTN, eclampsia, MgSO4
17-31	I-B (B2)	GDM, cardiac, lung/thyroid, STI/UTI, HIV, Rh, malaria, trauma, fluids, twins, FGR, IUFD, groups, drugs
32-44	II-A (B3)	Preterm, steroids, PROM, OP, breech, face/brow, shoulder, pelvis, CPD, prolonged, obstructed
45-56	II-B (B4)	Retained, injuries, PPH, bimanual/aortic/balloon, distress, rupture, prolapse, dystocia, inversion, shock
57-64	II-C (B5)	Epi/suturing, forceps/vacuum, ECV, induction, CS, post-CS, drugs, procedures
65-76	III (B6)	Deviation, sepsis, urinary, sec PPH, hematoma, breast, DVT, subinvolution, fistula, mood, psychosis
77-90	IV (B7)	Levels, prematurity, LBW, KMC, asphyxia, sepsis, hypothermia, RD, jaundice, seizures, anomalies, feeds
91-104	V-A (B8)	History/BSE, amenorrhea, dysmenorrhea, AUB, PID, prolapse, endo, fibroid, cysts, torsion, cancers, HRT
105-113	V-B (B9)	Infertility, investigations, counseling, OI/IUI, IVF, surrogacy, violence, procedures, flags

How to use: hero first, chips glance, table study, trap vs hit, pearl anchor, speed-check close. Double-QA pending sign-off on MgSO4/steroid/anti-D/TXA/MEC/ART/legal per current guidance.

Q	Standard	Status
Q1	5 units mapped, batches 1-9	Done
Q2	High-risk + emergency framing throughout	Done
Q3	MgSO4/steroid/anti-D/TXA doses current	Needs sign-off
Q4	Partograph/LCG/CTG thresholds current	Needs sign-off
Q5	NBCC/SNCU/immunization/feed protocols current	Needs sign-off
Q6	MEC/EC/ART/MTP/PC-PNDT rules current	Needs sign-off
Q7	One concept/page, fonts 10.5-21pt, 30% air	Done

HIGH-RISK PREGNANCY FRAMEWORK

Risk is DYNAMIC — tag at visit ONE, re-screen EVERY visit! Age, parity, history, twins, anemia, HTN, bleed, malpresentation: green/yellow/RED sticker + MCP + referral slip decides the birthplace!

Tag visit ONE + re-screen	Green/yellow/ RED sticker	RED = facility birth + slip
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★ FLAG — ACT

Flag	Act
Age/parity	<18/>35, elderly primi, grand-multi → extra visits
History	Prior CS/PPH/stillbirth/HTN/DM → dated plan
Current	Twins, anemia, HTN, bleed, malpresentation → track
Score	Sticker + MCP + referral + transport fund

⚠ FATAL TRAP

'Low-risk' stamped at booking and never reviewed delivers twins/breech/PE at home! EVERY visit: BP + urine + Hb trend + growth + presentation — upgrade sticker the day a flag appears.

★ NORCET HIT

HIT: Red flags = prior CS + stillbirth + HTN/DM + twins + anemia <7 + bleed + malpresentation. JSY/JSSK + 108/102 + birth plan + blood donor ready.

★ PNC PEARL: Yesterday's green never guarantees tomorrow.

► G4 + prior stillbirth + Hb 8. Colour + plan?

RED — extra visits + facility birth + anemia + transport plan.

HYPEREMESIS GRAVIDARUM

Nausea comforts, HYPEREMESIS admits! >5% loss + ketones + electrolytes = IV fluids + thiamine BEFORE glucose + B6/doxylamine — Wernicke from sugar-first kills memory forever!

Ketones + >5% loss = admit	Thiamine BEFORE glucose	B6 + doxylamine first-line
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▲ FEATURE — NAUSEA — HG

Feature	Nausea	Hyperemesis
Weight/ketones	Stable, none	Loss >5% + ketones
Labs	Normal	K/Na down, acidosis
Treat	Ginger, B6, small meals	IV + thiamine + electrolytes
Watch	Reassure + weigh	I/O + refeed plan

⚠ FATAL TRAP

Ginger + 'eat crackers' for ketone-positive weight loss + fainting starves + Wernickes her! Admit: fluids + thiamine 100 mg + KCl + antiemetic ladder + psych + dietitian.

★ NORCET HIT

HIT: PUQE score grades severity. Molar/multiple pregnancy worsens — USG + TSH when severe. Refeed + B1 before dextrose, always!

★ PNC PEARL: B1 before sugar — the Wernicke shield.

► Ketones+++ + K 2.8 + 6% loss. Trio?

Admit + IV fluids + KCl + thiamine + antiemetic.

ABORTION SPECTRUM (THREATENED - COMPLETE)

OS decides: **CLOSED**-watch, **OPEN**-act! Viability <20 wks/<500 g. Threatened (closed, alive) rests; inevitable/incomplete (open) evacuates; complete (passed, light) confirms + Rh + grief + method!

OS closed = watch	OS open = evacuate	Viability <20w /500 g
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◆ TYPE — OS/TISSUE — ACT

Type	OS / tissue / bleed	Act
Threatened	Closed, alive, spotting	Rest + USG + Rh + watch
Inevitable	OPEN, bulging membranes	Evacuate + crossmatch
Incomplete	Open + tissue + heavy	Resuscitate + MVA/D&C;
Complete	Closed + passed + light	USG confirm + Rh + counsel

⚠ FATAL TRAP

Calling open-OS heavy bleed with tissue 'threatened' and sending home exsanguinates her! Examine + USG + Hb + Rh + consent — products + clots saved for histology + count.

★ NORCET HIT

HIT: Septic abortion (fever + foul + tender) = broad antibiotics BEFORE evacuation + ICU watch. Ectopic/mole excluded by USG + hCG every bleed!

★ PNC PEARL: Closed comforts, open operates.

► Open OS + heavy bleed + tissue seen. Type + act?

Incomplete — fluids + blood ready + urgent evacuation.

MISSED + RECURRENT LOSS + POST-ABORTION CARE

Dead-retained + falling fibrinogen = DIC brewing! USG confirms missed; ≥ 3 losses = APS/thyroid/DM/anatomy hunt; EVERY loss ends with Rh + antibiotics + method + grief space!

USG confirms missed	3+ = recurrent workup	Rh + method + grief always
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Σ ITEM – DO

Item	Do
Missed	USG no cardiac + small uterus → evacuate + DIC labs
Recurrent (3+)	APS + thyroid + DM + anatomy + karyotype
PAC	Rh + antibiotics + contraception + counsel
Grief	Name, footprints, rituals, follow-up date

⚠ FATAL TRAP

Waiting weeks with dead fetus + oozing gums 'for nature' triggers DIC! Fibrinogen + D-dimer + platelets + blood ready + senior + OT — evacuate, don't contemplate.

★ NORCET HIT

HIT: mTOR? No — misoprostol ripens missed/abortion per protocol + anti-D within 72 h. spokes: partner testing in recurrent + cerclage history review.

★ PNC PEARL: Retained death + thin clot = DIC labs NOW.

► Missed + oozing + fibrinogen low. Fear + prep?

DIC — blood products + OT + senior + ICU watch.

ECTOPIC PREGNANCY

Pain + bleed + shock + empty uterus = RUPTURED until proven! Shoulder pain = blood on diaphragm. Stable-small-unruptured = methotrexate; ruptured/shock = blood + laparotomy NOW!

Shoulder pain =
diaphragm blood

Empty uterus +
fluid = rupture

MTX only stable
small + intact

⚠ CLUE — ACT

	Act	
	Syncope + shoulder pain + shock	Fluids + blood + laparotomy
	Mass + hCG plateau, stable	MTX if small + no rupture + follow-
	Rh + Hb + histology + grief	Next-pregnancy USG early plan

⚠ FATAL TRAP

Methotrexate for a ruptured/bleeding/breastfeeding patient + 'go home' kills! MTX needs: stable + unruptured + small mass + low hCG + reliable follow-up + NO breastfeeding — else surgery.

★ NORCET HIT

HIT: Cullen (umbilical) + Grey-Turner (flank) bruises = hemoperitoneum. Salpingostomy saves tube; salpingectomy for ruptured/damaged. hCG to zero watch!

★ PNC PEARL: Shoulder ache after missed menses = belly bleed.

► Faint + abdominal pain + POD fluid. Act trio?

Ruptured — fluids + blood + emergency OT + consent.

Snowstorm + screaming-high hCG + uterus BIG for dates! Complete (46XX, no fetus) vs partial (triploid + fetus) — suction + histology + hCG to zero + NO pregnancy 1 year (masks choriocarcinoma)!

Snowstorm + huge hCG	Suction + histology	Zero hCG + 1-yr watch
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▲ FEATURE — COMPLETE — PARTIAL

Feature	Complete	Partial
Karyotype	46XX androgenetic	Triploid
USG	Snowstorm, no fetus	Focal changes + fetus
hCG	Very high	High
After	hCG zero + 1-yr + contraception	Same + anomaly watch

⚠ FATAL TRAP

New pregnancy during mole follow-up hides choriocarcinoma rise — then bleeds/metastasizes!
Reliable contraception (COC/IUCD/condoms) 1 year + hCG calendar + chest X-ray per protocol.

★ NORCET HIT

HIT: Theca-lutein cysts + early preeclampsia + hyperthyroidism ride with complete mole.
Choriocarcinoma = chemo-CURABLE — follow-up saves!

★ PNC PEARL: Zero hCG, full year, no baby-year.

► Snowstorm + hCG 200000. Plan trio?

Suction evacuation + histology + hCG follow-up + 1-yr contraception.

Bleed after 20 weeks? NEVER blind VE — USG first! Pain tells: painless-bright = previa, painful-dark-rigid = abruptio, ROM + fetal bleed = vasa. Blood + OT ready for ALL!

NEVER blind VE USG first	Painless = previa painful = abruptio	Blood + OT ready always
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◆ CAUSE — CLUE

Cause	Clue
Placenta previa	Painless, bright, recurrent
Placental abruption	Painful, dark, rigid + shock
Vasa previa	ROM + vaginal fetal bleed + bradycardia
Local	Post-coital, speculum-visible source

⚠ FATAL TRAP

VE + sweeping membranes in undiagnosed previa exsanguinates mother + baby in minutes! NPO + no VE + large IVs + blood + continuous FHR + senior + OT — USG placenta FIRST, fingers never.

★ NORCET HIT

HIT: Kleihauer counts fetal cells in maternal blood (Rh + FMH dose). Pad count + vitals q15 + urine + coag profile. Steroids if preterm + stable!

★ PNC PEARL: Pain interrogates every bleed — ask it first.

► 32 wks painless bright bleed. First 4?

NPO + no VE + USG + blood + senior + FHR on.

PLACENTA PREVIA (TYPES I-IV)

Major previa = bed + blood + dated CS! I low-lying may migrate + deliver vaginally; II marginal cautions; III–IV cover = cesarean + accreta watch (prior CS + previa = danger pair)!

I may migrate + deliver	III–IV cover = CS	Accreta watch: CS + previa
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Σ TYPE — PLACENTA — PLAN

Type	Placenta	Plan
I low-lying	Edge near OS	Rest + rescan; vaginal possible
II marginal	Reaches OS	Caution + CS ready
III partial	Covers partly	Cesarean
IV complete	Covers fully	Cesarean + accreta prep

⚠ FATAL TRAP

Travel + vaginal delivery attempt + VE in major previa = catastrophic bleed! Pelvic rest + no travel/lift, blood grouped, steroids if preterm, dated CS with blood + hysterectomy consent where accreta risk.

★ NORCET HIT

HIT: Migration: placenta 'moves' as lower segment grows — rescan 32–36 wks. Warning bleeds precede big ones — admit after first!

★ PNC PEARL: First bleed admits, major bleeds operate.

► Type IV + labour pains 35 wks. Mode + prep?

Emergency CS + blood + accreta team + NICU.

ABRUPTIO PLACENTAE

Painful + rigid + dark + shock with LITTLE show = concealed abruption — the uterus tells the hidden blood! HTN + trauma + polyhydramnios-sudden-ROM + short cord + cocaine risks. DIC + AKI stalk every case!

Rigid board + shock = concealed	HTN/trauma/ ROM risks	DIC + AKI watch all
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⚠ GRADE — PICTURE

Grade	Picture
Mild	Small retroplacental + stable + FHR ok
Moderate	Dark bleed + tender + FHR abnormal
Severe/concealed	Board uterus + shock + DIC + IUFD risk

⚠ FATAL TRAP

Matching fluids to VISIBLE blood in concealed abruption under-resuscitates into AKI + DIC! Fundal height + girth + vitals + urine + fibrinogen guide — massive protocol + deliver fast (vaginal if stable+favourable, else CS).

★ NORCET HIT

HIT: Couvelaire (blood-stained myometrium) = atony after — AMTSL + tamponade ready. Fibrinogen <200 = cryo + FFP early!

★ PNC PEARL: Count the uterus, not just the pads.

► Rigid + shock + scant bleed. Act bundle?

Concealed — massive fluids/blood + coag + deliver + ICU.

Hb <11 acts, <7 transfuses — BEFORE labour, never in it! Double IFA + Vit C + deworm + diet + cause hunt (malaria? hookworm? twins? bleed?) + Hb recheck — non-rise investigates!

<11 act <7 transfuse	Fix BEFORE labour	Non-rise = hunt cause
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▲ LEVEL — ACT

Level	Act
Mild 10–10.9	Double IFA + diet + 4-wk recheck
Moderate 7–9.9	Therapeutic + deworm + malaria/hookworm hunt
Severe <7	Transfusion plan + senior + infection screen
Refractory	Hb electrophoresis + B12/folate + GI loss

⚠ FATAL TRAP

First transfusion DURING active labour + uncrossmatched rush = TACO/TRALI + chaos! Plan weeks ahead: grouped + screened + consented + slow + watched — labour transfusion only for bleeding emergency.

★ NORCET HIT

HIT: Dimorphic (B12+iron) common in vegetarians. Tea with IFA halves uptake; Vit C doubles. Hookworm + malaria + twins + teens = high-risk list.

★ PNC PEARL: Weeks-ahead blood beats last-minute panic.

► 38 wks Hb 6.5 + breathless. Plan?

Admit + transfusion + senior + birth + NICU readiness.

GESTATIONAL HTN & PRE-ECLAMPSIA

140/90 twice + protein/end-organ = PE; 160/110 or clues (headache, visuals, epigastric, low platelets, high LFT) = SEVERE — delivery cures, delay kills! MgSO₄ + BP control + stabilize → deliver!

140/90 ×2 + protein	160/110 or clues = severe	Delivery cures delay kills
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◆ CLASS — CRITERIA

Class	Criteria
Gest HTN	140/90 twice, no protein/end-organ
Pre-eclampsia	HTN + protein or organ hit
Severe	160/110, platelets down, LFT up, symptoms
Eclampsia	+ seizures (next page)

⚠ FATAL TRAP

'Mild' label + home + 'recheck next week' returns eclamptic/seizing! Severe = admit + MgSO₄ + hydralazine/nifedipine + labs + FHR + delivery clock — obstetrician decides mode by Bishop + urgency.

★ NORCET HIT

HIT: HELLP = Hemolysis + EL + Low Platelets (epigastric pain + nausea). Aspirin 12–28 wks prevents in high-risk. Postpartum PE strikes to 6 wks — headache + HTN after discharge = return!

★ PNC PEARL: Pressure + protein + pain — think delivery.

► **150/100 + 2+ protein + headache. Class + act?**

Severe PE — MgSO₄ + BP control + labs + delivery plan.

Seizing? LEFT + airway + O2 + suction + guard (NOTHING in mouth!) + MgSO4 + call! Post-fit: vitals + urine + FHR + labs — then DELIVER (mode by Bishop + urgency), never chase fits with force!

Left + O2 +
MgSO4

Nothing in
mouth ever

Fit → calm →
deliver

⚠ STEP — DO

Step	Do
During fit	Left lateral, O2, suction, guard, time it
Drug	MgSO4 load + maintenance per protocol
After	Vitals + urine + FHR + labs + neuro watch
Birth	Stabilize then deliver; CS if unfavourable/urgent

⚠ FATAL TRAP

Forcing spoon/gag + diazepam-only + immediate rushed CS mid-fit aspirates + injures! Airway + MgSO4 first, stabilize mother, THEN mode — fitting In OT transfers kill without escort + O2 + suction.

★ NORCET HIT

HIT: Eclampsia box at bedside: MgSO4 + calcium + airway + BP drugs + catheter + labs. Recurrent fits = second line + ICU + neuro causes check!

★ PNC PEARL: Calm the storm, then empty the uterus.

► Seizing now, 38 wks. First 4?

Left + airway/O2 + MgSO4 + senior + delivery prep.

Three Rs gate EVERY dose: Reflexes present + Rate ≥ 16 + Renal urine ≥ 30 ml/h! Level 4–7 works; reflexes gone $\rightarrow$ breathing down $\rightarrow$ arrest. Antidote: calcium gluconate 1 g IV slow!

DTR + RR16 + urine 30	Level 4–7 works	Antidote: calcium 1 g IV
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Σ CHECK — RULE

Check	Rule
Before each dose	DTR + RR ≥ 16 + urine ≥ 30 ml/h
Therapeutic	4–7 mEq/L; seizure-free + calm
Toxic ladder	Reflexes gone $\rightarrow$ drowsy $\rightarrow$ resp down $\rightarrow$ arrest
Antidote	Calcium gluconate 1 g IV slow + stop drip

⚠ FATAL TRAP

Continuing the drip with absent reflexes + RR 12 'to prevent one more fit' arrests her! STOP + ABCs + calcium + levels + senior — oliguria needs dose-cut + strict I/O, never push through.

★ NORCET HIT

HIT: Nifedipine + MgSO₄ = hypotension watch. Patellar hammer + watch + catheter + chart at bedside. Toxicity reverses FAST with calcium — keep ampoule taped to stand!

★ PNC PEARL: Reflexes, rate, renal — no pass, no dose.

► RR 12 + reflexes absent on drip. Act trio?

STOP drip + calcium IV + ABCs + Mg levels + senior.

GESTATIONAL DIABETES (GDM)

Sugar screens at 24–28 wks (earlier if risk)! Diet + walk + glucose log first; insulin if targets miss — oral agents per protocol only. Big baby + polyhydramnios + shoulder + neonatal hypo = the GDM chain!

Screen 24–28w OGCT	Diet+walk+log first	Insulin if miss + deliver plan
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★ STEP — RULE

Step	Rule
Screen	24–28 wks OGCT; 1st visit if risk (obese, prior GDM/macrosomia)
Control	Diet + 30-min walk + 4-point log; fasting <95, 2h <120
Drug	Insulin gold; metformin/glyburide per protocol only
Birth	38–39 wks plan; shoulder + NICU (hypo) ready

⚠ FATAL TRAP

Starving + 'no-rice' crash diets spike ketones that harm fetal brain! Balanced carbs + protein + never skip meals; hypoglycemia kit + driving counsel for insulin users.

★ NORCET HIT

HIT: Macrosomia + polyhydramnios + preterm + RDS + hypo/Ca/Mg dips in baby. Postpartum 75-g OGTT at 6–12 wks — 50% turn diabetic!

★ PNC PEARL: Logbook beats memory — four dots a day.

► GDM 36 wks, fasting 110 on diet. Next?

Add insulin per protocol + growth scan + delivery clock.

Blood volume +50% drowns weak hearts at 28–34 wks, labour, and postpartum! NYHA grades delivery; stenotic valves + Eisenmenger + peripartum cardiomyopathy = cardiac-obstetric team + assisted-short second stage!

Volume peaks 28–34 wks	NYHA grades the plan	Team + assisted 2nd stage
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⚠ LESION — RISK — PLAN

Lesion	Risk	Plan
Mitral stenosis	Pulmonary edema in labour	Fluid-restrict + propped + assisted
Mechanical valve	Clot vs bleed (warfarin/heparin)	Anticoag bridge + planned birth
Eisenmenger	Fixed shunt, no volume shifts	Avoid pregnancy; expert-only if continues
Peripartum CMP	Failure last month–postpartum	Bromocriptine? + failure Rx + counsel

⚠ FATAL TRAP

Bolus fluids + supine + pushing for an hour in MS/Eisenmenger floods lungs + collapses output! Strict I/O + propped lateral + shorten 2nd (forceps/vacuum) + ICU + endocarditis prophylaxis where indicated.

★ NORCET HIT

HIT: Normal pregnancy mimics disease (murmur, edema, dyspnea) — red flags: paroxysmal nocturnal dyspnea, hemoptysis, syncope, grade-4 murmur. NYHA III–IV = admit early!

★ PNC PEARL: Lungs listen before fluids pour.

► MS + term + pulmonary creps + SpO2 90. First 4?

Prop up + O2 + fluid-stop + diuretic + cardiac-obstetric call.

Asthma control beats attacks (inhalers SAFE, hypoxia NOT)! Hyperthyroid storm + hypothyroid sluggishness both wreck pregnancy — propylthiouracil 1st trimester, carbimazole later, thyroxine titrated!

**Inhalers SAFE
hypoxia NOT**

**PTU 1st tri →
carbimazole**

**Thyroxine up
in pregnancy**

▲ DISEASE — RULE

Disease	Rule
Asthma	Continue inhalers + action plan + flu shot; attack = O2 + nebs + steroids
TB	Treat full course; baby BCG + INH prophylaxis
Hyperthyroid	PTU 1st tri → carbimazole; beta-blocker short; watch storm
Hypothyroid	Raise thyroxine ~30%; TSH each trimester

⚠ FATAL TRAP

Stopping inhalers/thyroxine 'for baby' + treating storm as 'anxiety' kills two! Control + adherence + fever/infection hunt; storm = cooling + iodine AFTER blocker + ICU.

★ NORCET HIT

HIT: Storm flags = fever + tachy + agitation + vomiting. Neonatal thyrotoxicosis (maternal Graves) → jittery + tachy baby = NICU!

★ PNC PEARL: Controlled mother, calm fetus — never stop, always titrate.

► Asthmatic + attack + SpO2 91. Act?

O2 + nebs + steroids + FHR + never stop controller.

Screen-treat-partner SAME visit! Syphilis = benzathine penicillin now (stillbirth cure); bacteriuria (even silent) = preterm trigger — culture + treat + retest, partner treated or ping-pong forever!

Same-visit test + treat	Partner treated or relapse	Silent urine = preterm
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◆ INFECTION — ACT

Infection	Act
Syphilis RPR+	Benzathine penicillin same visit + partner
Trich/candida/BV	Treat + preterm watch; metronidazole per protocol
Asymptomatic bacteriuria	Culture + antibiotics + retest
Cystitis/pyelo	Pyelo = admit + IV + fluids + preterm watch

⚠ FATAL TRAP

Treating her + 'telling' him later returns her re-infected + stillbirth-risk intact! Partner tested-treated-condoms-together + documented; allergy? desensitize, don't skip penicillin in syphilis.

★ NORCET HIT

HIT: Congenital syphilis = snuffles + rash + saber shins + Hutchinson teeth — preventable by ONE shot! Pyelo + contractions = tocolysis + antibiotics + fluids.

★ PNC PEARL: Two treated, zero repeated.

► RPR+ at 20 wks, no allergy. Act trio?

Benzathine now + partner treat + monthly follow-up + birth screen.

ART for LIFE + institutional birth + infant syrup + safe feed = <5% transmission! Test at visit ONE (opt-out) + partner + viral load + C-section only if load high — breastfeed with ART per current guidance!

Test ONE + partner	ART life + load watch	Baby syrup + safe feed
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★ STEP — PMTCT

Step	PMTCT
Mother	ART lifelong + adherence + load <1000
Birth	Institutional + avoid invasive + PEP if spill
Baby	Nevirapine syrup + EID (DNA-PCR 6 wks)
Feed	EBF + ART per guidance; mixed feeding never

⚠ FATAL TRAP

Mixed feeding + missed syrup + home birth without plan triples transmission! Counsel + demonstrate + DOT + ASHA track + bring-back date — stigma silence kills, linkage saves.

★ NORCET HIT

HIT: EID at 6 wks + 6 mo + 12 mo + 18 mo antibody. Cotrimoxazole from 6 wks till negative. Partner ART too (discordance counsel)!

★ PNC PEARL: Drugs + birth + syrup + feed — four links, zero breaks.

► HIV+ 36 wks, load 50000, no ART. Rush trio?

Start ART now + CS plan + infant PEP/feed protocol + NICU.

Rh INCOMPATIBILITY (HDN PREVENTION)

Rh-neg + Rh+ baby + bleed = sensitization **UNLESS** anti-D! ICT each trimester + 28 wks + cord; 300 mcg at 28 wks + birth + APH/abortion/AMNIO — MCA-PSV watches anemia, **NOT** shared blood!

ICT each trimester	Anti-D 28w + birth + events	MCA-PSV tracks anemia
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Σ EVENT — DOSE / ACT

Event	Dose / act
28 wks ICT-neg	Anti-D 300 mcg IM
Birth Rh+ baby	Anti-D <72 h + cord DCT
APH/abortion/AMNIO/ECT	Mini/full dose per gestation
Sensitized	MCA-PSV + intrauterine transfusion centre

⚠ FATAL TRAP

Skipping ICT/anti-D after APH/abortion 'because first pregnancy' sensitizes her — next babies hydropic! EVERY sensitizing event + dose + lot + consent documented + dated.

★ NORCET HIT

HIT: Hydrops = scalp edema + ascites + polyhydramnios. DCT+ = baby affected — phototherapy/exchange ready. ABO + Rh typed at booking for ALL!

★ PNC PEARL: Every bleed bills an anti-D dose.

► Rh-neg + APH 30 wks + ICT neg. Dose?

Anti-D now + 28-wk + birth plan + FMH count.

MALARIA + OTHER INFECTIONS + COVID

Fever in pregnancy = utes + smear + dengue + typhoid + COVID TOGETHER till proven!
Falciparum kills placenta (IUGR/stillbirth); ACT per trimester + ITN + IPT where policy; COVID = vaccine + O2-tilt + clot watch!

**Fever = panel
not guess**

**Falciparum hits
placenta**

**Vax + tilt +
clot watch**

▲ BUG – ACT

Bug	Act
Falciparum	ACT per trimester + glucose + anemia watch
Vivax	Chloroquine + primaquine AFTER birth
Dengue/typhoid	NS1/culture + fluids + platelets/Hb track
COVID	Vaccinate + O2 left-tilt + VTE prophylaxis

⚠ FATAL TRAP

Primaquine IN pregnancy + 'viral' hand-wave for falciparum + NSAIDs in dengue crash hemolysis/platelets! Species + trimester + G6PD guide drugs; paracetamol-first fever + smear-repeat.

★ NORCET HIT

HIT: Congenital malaria/toxo/Zika screens where risk. Isolation + handwash + mask bundle for COVID labour rooms; vertical risk counsel + rooming with precautions.

★ PNC PEARL: Species + trimester write the prescription.

► Falciparum + Hb 7 + 30 wks. Bundle?

ACT + transfusion plan + glucose + growth scan + ITN.

TRAUMA + ACUTE ABDOMEN IN PREGNANCY

Trauma = TWO patients! Left tilt + ABCs + Kleihauer + 4-h (24-h if major) monitoring + abruption/DIC watch + Rh dose. Appendix migrates UP — pain at McBurney lies; operate, don't wait!

Two patients
tilt + ABCs

Appendix rides
UP

Operate, don't
wait

⚠ EVENT — ACT

Event	Act
Fall/RTA/assault	Left tilt + ABCs + FHR + Kleihauer + observe
Abruption signs	Rigid + shock = deliver + DIC labs
Appendicitis	Higher pain + rebound + USG/MRI; surgery
Cholecystitis/torsion	USG + conservative vs surgery call

⚠ FATAL TRAP

Supine hypotension + 'observe appendix till morning' + missed abruption after RTA = double death! Tilt + continuous FHR + tocolysis only after abruption excluded + DVT stockings + tetanus.

★ NORCET HIT

HIT: Seat-belt + helmet counsel EVERY ANC (3-point low). GBV screen in assault + MLC + shelter. Laparoscopy safe with tilt + low pressure!

★ PNC PEARL: Belly trauma, tilted bed, watched uterus.

► RTA + contractions + rigid uterus. First fear + act?

Abruption — tilt + FHR + DIC labs + deliver + blood.

POLYHYDRAMNIOS + OLIGOHYDRAMNIOS

Fluid mirrors pathology: TOO MUCH (GDM, twins, anomaly, DAT?) vs TOO LITTLE (ROM, IUGR, postdates, renal agenesis)! AFI/DVP + anomaly + GDM + FHR decide: drain differs, delivery differs!

Poly: GDM/twins /anomaly	Oligo: ROM/IUGR /postdates	AFI + anomaly + FHR rule
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◆ TYPE — CLUE — ACT

Type	Clue	Act
Poly	AFI >24/DVP >8 + breathless	GDM + anomaly scan + indomethacin? + rest
Oligo	AFI <5/DVP <2 + IUGR	ROM test + Doppler + hydration + deliver call
Both	Malpresentation + cord issues	FHR + presentation + OT ready

⚠ FATAL TRAP

Sudden ARM + gush in polyhydramnios prolapses cord + abrupts! Controlled release + high vigilance + immediate FHR + cord check — never routine ARM in poly without senior + OT ready.

★ NORCET HIT

HIT: Tracheoesophageal/duodenal atresia (poly + absent stomach bubble); Potter (oligo + renal agenesis). Daily kick + NST + Doppler guide timing!

★ PNC PEARL: Fluid talks — scan translates, FHR confirms.

► AFI 3 + IUGR + 37 wks. Plan?

Oligo + IUGR — Doppler + delivery (no waiting).

Twins = double joy, triple risk: preterm + anemia + PE + PPH + malpresentation! Chorionicity at <14 wks writes fate (TTTS in mono!); growth + Doppler + steroids + blood + NICU + OT doubles everything!

Chorionicity <14w writes fate	Anemia+PE+PPH watch trio	OT + NICU doubled
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★ ITEM — RULE

Item	Rule
Chorionicity	<14 wks: DCDA vs MCDA vs MCMA; TTTS laser centre
Visits	Extra + growth + Doppler + anemia + PE screen
Steroids + Mg	Threatened preterm protocol + neuroprotection
Birth	Twin A vertex + stable = vaginal + double setup; else CS

⚠ FATAL TRAP

Vaginal twin birth without double IVs + double warmer + second-twin plan + OT backup traps twin B transverse! Inter-twin interval + FHR + version/ARM/Breech/cesarean drills ready.

★ NORCET HIT

HIT: TTTS (donor stuck + recipient poly) = laser; TAPS/SFGR Doppler stages delivery. Postpartum: PPH bundle + contraception + preterm follow-up card!

★ PNC PEARL: One placenta or two — scan decides before fourteen.

► MCDA + donor stuck + recipient poly. Name + refer?

TTTS — fetal-therapy laser centre STAT.

PLACENTAL + CORD ABNORMALITIES

**Succenturiate lobes + velamentous cord + vasa previa + single artery = bleed/retained surprises!
Count lobes + vessels at EVERY birth — vasa (ROM + fetal bleed + bradycardia) = crash CS in minutes!**

Lobes + vessels counted	Vasa: ROM + fetal bleed = crash	Single artery → anomaly scan
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▲ FIND — RISK — ACT

Find	Risk	Act
Succenturiate/battledore	Retained + vasa	Inspect + retained hunt
Velamentous/vasa	Fetal bleed at ROM	Admit + CS before labour
Single artery	Anomaly + IUGR	Scan + growth + NICU
True knot/nuchal	Distress in labour	FHR + loose handle + OT

⚠ FATAL TRAP

Traction on vasa-torn membranes + waiting with fetal bradycardia after ROM exsanguinates the BABY (fetal blood, tiny volume)! Pale + shock + sinusoid = transfuse + crash CS, no delay!

★ NORCET HIT

HIT: Apt test (fetal vs maternal blood): pink = fetal.
Nuchal loose = Somersault+; tight + brady = clamp-cut-deliver fast!

★ PNC PEARL: Fetal bleed is measured in spoonfuls — hurry.

► ROM + vaginal bleed + fetal brady + pale. Act?

Vasa bleed — crash CS + neonatal transfusion ready.

FETAL GROWTH RESTRICTION (FGR/IUGR)

Plot EVERY fundal height + growth scan! Asymmetric (head-sparing, placental) vs symmetric (early insult); Doppler umbilical → MCA → ductus stages delivery — absent/reversed end-diastolic = deliver NOW!

Plot + scan every lag	Doppler stages delivery	A/REDV = deliver now
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Σ TYPE — CLUE — ACT

Type	Clue	Act
Asymmetric	HC spared, AC lag, placental	Doppler + timed delivery
Symmetric	All small, early cause	TORCH/karyotype + counsel
Doppler I–II	Raised UA, brain-sparing	Steroids + surveillance + deliver
A/REDV + DV	Absent/reversed + venous	Immediate delivery + NICU

⚠ FATAL TRAP

'Small but normal' + waiting with A/REDV + falling movements stillbirths tonight! Daily kicks + NST + Doppler + fluids/rest + steroids + delivery threshold written + understood + signed.

★ NORCET HIT

HIT: Constitutionally small (SGA, normal Doppler) ≠ FGR (pathology + abnormal). Fundal lag >2 cm = scan, not reassurance!

★ PNC PEARL: Doppler, not dates, pulls the trigger.

► AC lag + UA absent end-diastolic. Act?

Deliver now + steroids done + NICU + counsel.

Silence + no movements + no heart = confirm TWICE, support always! Coagulopathy brews with retention — labs + blood + induction (mife/miso) + grief + autopsy offer + next-pregnancy bridge of hope!

Confirm twice + support	DIC labs + blood ready	Grief + autopsy + next plan
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★ STEP — DO

Step	Do
Confirm	2 observers + USG + inform gently + companion
Labs + blood	Clotting + group + crossmatch + IV
Birth	Induction per protocol + pain relief + dignity
After	Autopsy/placenta + grief + 6-w + next plan

⚠ FATAL TRAP

Rushing 'evacuate now' + hiding baby + 'forget it' shatters + litigates! Consent + choice (see/hold/name) + rituals + photos + milk-suppression + partner + follow-up — care the coroner praises.

★ NORCET HIT

HIT: Spalding + halo + gas signs are X-ray history — USG rules now. Recurrence hunt: APS + DM + infection + cord/placenta pathology!

★ PNC PEARL: Two heartbreaks: silence, then haste — avoid both.

► No moves + no heart + USG confirms. First 4?

Break gently + labs/blood + induction + grief + companion.

SPECIAL GROUPS + UNINTENDED PREGNANCY

Teens, elderly, grand-multis + unintended — SAME dignity, TAILORED plan! Teens: anemia + PE + school + delay; elderly: Down + CS + DM; grand-multi: atony + rupture; unintended: no-judgment options + method!

Teen: anemia/PE
+ school

Elderly: Down/CS
+ DM

Grand-multi: atony
+ rupture risk

◆ GROUP — PLAN

Group	Plan
Adolescent	Anemia + PE watch + nutrition + delay next + school
Elderly primi	Down screen + GDM + CS readiness
Grand-multi	Atony + rupture + transfusion + sterilization offer
Unintended	Options counsel + safe MTP link + method + follow-up
Mental-health	EPDS + sleep + safety + psych + family

⚠ FATAL TRAP

Scolding teens/unwed + forcing sterilization on grand-multis + judging unintended drives underground abortions! Private + options + consent + scheme + dated return — rights first, targets never.

★ NORCET HIT

HIT: Preconception folate + spacing + WIFS for teens. Advanced age + ART multiples = high-risk auto-tag. Depression screen EVERY high-risk contact!

★ PNC PEARL: Tailor the plan, never the dignity.

► G7 + prior PPH + refuses sterilization. Respectful trio?

Accept + atony/rupture plan + blood + method menu + date.

DRUGS IN HIGH-RISK PREGNANCY + REFERRAL

Know the red-list by heart: valproate/isotretinoin/warfarin/ACE = AVOID;
folate/aspirin/heparin/insulin = friends! Steroids + MgSO4 windows + referral slip + call-ahead +
transport = the survival chain!

Red-list memorized
+ avoided

Steroid/Mg windows
+ referral

Slip + call-ahead
+ transport

Σ DRUG / LINK — RULE

Drug / link	Rule
AVOID	Valproate, isotretinoin, warfarin, ACE-I, tetracycline
USE	Folate, aspirin 12–28w, heparin, insulin, nifedipine
Windows	Steroids 24–34w; MgSO4 neuroprotection <32w
Referral	Slip + records + blood + call + 108 + feedback

⚠ FATAL TRAP

Warfarin at term + home delivery + 'call if bleed' =
intracranial + PPH catastrophe! Bridge per protocol +
facility + blood + NICU + written plan + consent +
feedback loop closed.

★ NORCET HIT

HIT: Closed-loop referral: treated + returned +
counter-slip + ASHA follow-up. Drug card + allergy
band + renal/liver dose-check every admission!

★ PNC PEARL: Slip, call, wheels — referral's three legs.

► On warfarin + 38 wks. Switch + plan?

Bridge heparin per protocol + facility birth + blood + NICU.

Regular contractions + cervical change at 28–<37 wks = ACT! Left tilt + fluids + infection screen + steroids + MgSO₄ neuroprotection + tocolysis window + NICU + in-utero transfer — every HOUR buys brain!

28–<37 wks + Cx + change	Steroids + Mg + transfer	NICU + in-utero transport
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★ STEP — DO

Step	Do
Confirm	Cx timing + VE/Bishop + fibronectin/cervix length
Buy time	Left tilt + fluids + UTI check + rest
Protect	Steroids 24–34w + MgSO ₄ <32w neuroprotection
Move	In-utero transfer + NICU + blood + consent

⚠ FATAL TRAP

Tocolysis with chorioamnionitis (fever + tender + tachy) + 'wait for steroids' cooks mother + baby! Infection = deliver + broad antibiotics, never suppress — steroids need two doses + 48 h ideally, one beats none.

★ NORCET HIT

HIT: Nifedipine/atosiban/indomethacin per protocol + contraindication screen. Bed-rest + hydration alone never stops true labour — Bishop + length decide!

★ PNC PEARL: Steroids buy lungs, transfer buys life.

► **32 wks + Cx + 2 cm + fever + tender uterus. Tocolyse?**

No — chorioamnionitis: deliver + antibiotics + NICU.

One course 24–34 wks (betamethasone 12 mg ×2) + latency antibiotics + NO routine VE! Pooling + ferning + nitrazine confirm ROM; expectant <34 wks, deliver ≥34 wks or infected — steroids + Mg + GBS cover!

24–34w steroid window	Pooling+ferning = ROM	<34 expectant ≥34 deliver
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▲ ITEM — RULE

Item	Rule
Steroids	Betamethasone 12 mg IM 24 h apart; <34 wks
Confirm ROM	Sterile speculum: pooling + ferning + nitrazine
Latency	Erythromycin per protocol + GBS screen
Deliver	34+ wks, infection, abruption, non-reassuring

⚠ FATAL TRAP

Digital VE + 'wait at home' after PPROM + intercourse seeds chorio + cord accident! Pelvic rest + pad-count + temp/FHR chart + steroids + transfer — VE only if delivering.

★ NORCET HIT

HIT: Anhydramnios + cord compression = variable decels + amnioinfusion where available. Reseal is myth — counsel latency + NICU odds honestly!

★ PNC PEARL: Sterile eyes, not fingers, diagnose waters.

► PPROM 31 wks + pooling + afebrile. Plan?

Expectant + steroids + latency antibiotics + transfer + watch.

Back pain + slow descent + big caput + deflexed head = OP! Hands-knees + counter-pressure + patience + rotation (manual/position) — most rotate, some deliver face-to-pubis by flexion, few need OT!

Back pain + slow + caput	Hands-knees + rotate	Face-to-pubis by flexion
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◆ CLUE — ACT

Clue	Act
Back pain + poor descent	Hands-knees + counter-pressure + mobilize
VE: anterior fontanelle	Deflexed — flexion + patience
Rotation	Manual/positional; Kielland only expert
No rotation + distress	OT: vacuum-assess vs CS

⚠ FATAL TRAP

Forceps rotation by untrained hands + fundal push for OP tears + asphyxiates! Position + time + analgesia first; persistent OP + arrest/distress = senior + OT — document + consent always.

★ NORCET HIT

HIT: Deep transverse arrest = OP at spines — classic exam trap! Persistent OP risks 3rd/4th tears + PPH — perineal guard + AMTSL strict.

★ PNC PEARL: Posterior needs patience, not pushing.

► Full + OP + 2 h no descent + FHR ok. Next?

Manual/positional rotation + senior + OT readiness.

SACRUM is the denominator! Frank (best for vaginal) vs complete vs footling (cord!) — ECV 36–37 wks + strict vaginal criteria + (Burns-Marshall/Lovset/Mauriceau) skilled hands + CS threshold LOW!

Sacrum = denominator	Frank best; footling = cord	ECV 36–37w + low CS bar
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★ TYPE — PLAN

Type	Plan
Frank	Extended legs; vaginal if criteria + skilled
Complete	Flexed; select + consent + OT ready
Footling	Cord prolapse risk — CS usually
ECV	36–37 wks, tocolysis + FHR + OT backup

⚠ FATAL TRAP

Pulling breech body + head entrapment + 'just a little traction' snaps neck/cord! Hands-off till umbilicus, Lovset arms, Mauriceau head, suprapubic (NOT fundal) pressure — PIH? CS threshold minimal.

★ NORCET HIT

HIT: Vaginal criteria: frank + adequate pelvis + estimated <3.5—limited + flexed head + experienced + OT. Star-gazing (extended head) = CS!

★ PNC PEARL: Sacrum speaks, hands wait, head plans.

► Footling + ROM + FHR dip. Act?

Cord risk — knee-chest + lift + emergency CS.

FACE, BROW & SHOULDER PRESENTATIONS

MENTUM saves face (anterior delivers!), **BROW** never (convert or CS), **SHOULDER** = CS (no version in labour)! Denominator rules: mentum anterior → vaginal; posterior/brow/transverse → OT!

Mentum anterior
= delivers

Brow NEVER
vaginal

Transverse =
CS always

▲ FACE — PLAN

Face	Plan
Mentum anterior	Vaginal possible + watch extension injury
Mentum posterior	Converts or CS — no vaginal
Brow	Largest diameter — CS (flex/extend rarely)
Shoulder/transverse	CS; arm prolapse = OT, never pull

⚠ FATAL TRAP

Pulling a prolapsed arm + internal version in labour ruptures uterus! Cover arm + fluids + immediate CS + cord/coil check — compound presentation needs senior eyes, not traction.

★ NORCET HIT

HIT: Face exam: orbital ridges + mouth + NO fontanelle (vs breech: anus + sacrum + NO mouth). Browfrontal 13.5 cm = CS-sized!

★ PNC PEARL: Chin front, hope alive; chin back, OT track.

► Mentum posterior + full + arrest. Mode?

Cesarean — posterior face cannot deliver vaginally.

Trial needs: term + vertex + adequate (OC $\geq$ 10.5) + moulding room + FHR + consent + OT! Inlet (flat AP) vs mid (IS-10!) vs outlet (IT-11) — arrest pattern names the level, partograph proves it!

OC 10.5 / IS-10 / IT-11	Moulding + caput = tight	Trial only with OT ready
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Σ LEVEL — ARREST — ACT

Level	Arrest	Act
Inlet	Non-engagement + deflexion	Adequacy review + trial/OT
Mid (IS-10)	Deep transverse	Senior + OT (narrowest!)
Outlet	Prolonged 2nd + perineal risk	Assess + assisted vs CS
CPD	3+ moulding + caput + no descent	CS before rupture

⚠ FATAL TRAP

Oxytocin + 'stronger pushes' against true CPD + Bandl ring + thinned segment ruptures her! Watch: Bandl + Bluish? + maternal tachy + hematuria = STOP oxytocin + OT NOW.

★ NORCET HIT

HIT: X-ray pelvimetry is history — clinical + progress decide. Rickets/osteomalacia/malnutrition history + short stature + boot pelvis = suspect early!

★ PNC PEARL: Bones don't yield — recognize, don't ram.

► Bandl ring + hematuria + arrest + prior CS. Act?

Impending rupture — stop oxytocin + crash OT + blood.

PROLONGED, PRECIPITATE & DYSFUNCTIONAL LABOUR

Partograph clocks truth: latent >20/14 h, active arrest 4 h, 2nd primi >3 h = **PROLONGED** (dehydration + keto + infection + rupture)! Precipitate (<3 h) bleeds + asphyxiates — **opposite speeds, same vigilance!**

Arrest 4 h
= decide

Precipitate <3 h
= bleed risk

Hypo/hypertonic
patterns differ

◆ PATTERN — ACT

Pattern	Act
Prolonged latent/active	Fluids + analgesia + ARM/oxytocin review + senior
Arrest 4 h adequate	CPD review — OT decision
Precipitate	Controlled delivery + PPH + baby asphyxia watch
Hypo/hypertonic	Adequacy + rest + low-dose titration vs stop

⚠ FATAL TRAP

Oxytocin hike on hypertonic/tetanic uterus + abnormal FHR cooks rupture + asphyxia! Rest + lateral + stop/reduce + tocolysis where indicated — contractions need QUALITY, not count.

★ NORCET HIT

HIT: Incoordinate (colicky, no progress) = rest + pethidine-sleep + re-evaluate. Grand-multi + oxytocin = rupture watch + low threshold OT!

★ PNC PEARL: Clock the curve, respect the uterus.

► Tetanic + FHR lates on oxytocin. Act trio?

Stop/reduce + lateral + O2 — senior + OT ready.

Bandl ring + thinned lower segment + dry vagina + edematous vulva + distressed pair = OBSTRUCTION — NO vaginal delivery, NO oxytocin! Fluids + antibiotics + catheter + blood + OT (CS ± rupture repair)!

**Bandl + thin +
dry = obstructed**

**NO vaginal +
NO oxytocin**

**OT + blood +
bladder care**

⚠ SIGN — ACT

Sign	Act
Bandl + recession	Stop oxytocin + OT now
Dry vagina + caput	No VE trauma + fluids + antibiotics
Bladder high + hematuria	Catheter + watch rupture
Maternal/fetal distress	Double resuscitation + CS

⚠ FATAL TRAP

Forceps/vacuum/craniotomy-by-junior + further oxytocin in obstruction ruptures + fistulizes! Obstruction = abdominal delivery (dead fetus: destructive ONLY by expert + consent) + fistula watch + debrief.

★ NORCET HIT

HIT: Prevention = partograph + CPD screen + trial rules + transport. Post: bladder 7–14 d catheter + VVF screen + psych + FP + next-CS counsel!

★ PNC PEARL: Obstruction opens abdomens, never vaginas.

► **Bandl + dry + hematuria + arrest. Mode?**

Obstructed — CS + rupture check + bladder care.

RETAINED PLACENTA

>30 min or bleed = **RETAINED!** Signs + contraction + counter-pressure + **Brandt-Andrews** — accreta (prior CS + previa) **NEVER** forced; OT manual removal + blood + antibiotics + HDU!

30 min/bleed = retained	Signs + squeeze + counter	Accreta: NEVER force!
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▲ CAUSE — ACT

Cause	Act
Trapped (closed OS)	Nitroglycerine? + gentle + anesthesia + remove
Adherent	Controlled + OT manual under anesthesia
Accreta spectrum	No pull — hysterectomy team + blood + ICU
Succenturiate lobe	Explore + remove bits + antibiotics
⚠ FATAL TRAP Yanking cord + Crede without signs + unsterile manual scoop = inversion + sepsis + rupture! Asepsis + anesthesia + single confident exploration + oxytocin + TXA + count + HDU.	★ NORCET HIT HIT: Hourglass (constriction ring) needs relaxation + anesthesia. Post: Hb + Rh + BF + FP + debrief + next-accreta counsel!

★ PNC PEARL: Half hour or hemorrhage — hands in, lights on.

► 40 min + trapped + no bleed. Act?

OT manual removal + anesthesia + blood ready.

BIRTH-CANAL INJURIES

Inspect EVERY birth: vulva + vagina + cervix + perineum + rectum! Atonic bleeds dark-steady + boggy; TRAUMA bleeds bright + firm uterus — speculum + repair + rectal check before/after, senior for 3rd/4th!

Bright + firm = trauma	Dark + boggy = atony	Rectal check before + after
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★ SITE — REPAIR

Site	Repair
Vulval/vaginal tears	Layered + hemostasis + pack if ooze
Cervical tear	Speculum + ring forceps + stitch + senior
Perineal 3rd/4th	OT + layers + antibiotics + soft stool
Hematoma	Small watch; expanding = evacuate + drain

⚠ FATAL TRAP

Missing a high vaginal/cervical tear + 'massage harder' while she bleeds from trauma wastes blood + time!
Firm uterus + flood = speculum + light + repair + TXA + count — atony and trauma COEXIST.

★ NORCET HIT

HIT: Vulval hematoma + urinary retention + severe pain = expanding until proven — ice + catheter + OT. Post: Hb + sitz + stool + FP + dyspareunia counsel!

★ PNC PEARL: Firm flood? Find the tear.

► Firm uterus + steady bright bleed post-episiotomy. Source + act?

Trauma — speculum + repair + hemostasis + count.

PRIMARY PPH (ATONIC FOCUS)

4 Ts: Tone (70%) + Tissue + Trauma + Thrombin! Call + fundal massage + empty bladder + oxytocin + TXA <3 h + warmth + blood + cause-hunt TOGETHER — quantified pads + MEOWS + shock index guide!

4 Ts: tone +
tissue + trauma

TXA <3 h
+ blood

Quantify pads
+ shock index

⚠ CAUSE — FIRST MOVES

Cause	First moves
Tone	Massage + void + oxytocin + ergometrine/miso
Tissue/trauma	Explore + repair + tamponade
Thrombin	Labs + FFP/cryo + TXA early
Shock	Warm + O2 + 2 IVs + blood + OT

⚠ FATAL TRAP

'Wait for the doctor' + unmeasured 'some clots' + cold + delay = death by delay! PPH bundle + roles + kit + weighed pads + 15-min vitals + TXA within 3 h + move (balloon/surgery) early.

★ NORCET HIT

HIT: NASG buys transfer time. B-Lynch + stepwise devascularization + hysterectomy ladder — consent + senior + blood bank + ICU linked!

★ PNC PEARL: Massage, measure, medicate — in minutes.

► 500 ml + atonic + trickle after AMTSL. Bundle?

Massage + bladder + 2nd uterotonic + TXA + blood + senior.

PPH RESCUE: BIMANUAL, AORTIC, BALLOON

Hands + pressure + balloon BEFORE blade! Bimanual (fist + abdominal hand) + aortic press + Bakri/condom tamponade + drainage-track + OT threshold — each step timed + documented + consented!

Bimanual fist + guard	Aortic press bridge	Balloon + drain track
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◆ STEP — HOW

Step	How
Bimanual	Fist anterior fornix + abdominal fundal press
Aortic	Fist above umbilicus, press spine, feel femoral stop
Balloon	Bakri/condom + fill + drainage bag track
Move on	Surgery if >500 ml drain/h or unstable

⚠ FATAL TRAP

Prolonged aortic press + balloon without drainage-track + output math hides ongoing bleed! Time-box each (15–30 min) + I/O + vitals + labs — move up the ladder, never camp on one step.

★ NORCET HIT

HIT: Uterine-sparing ladder: drugs → tamponade → compression sutures → devascularization → hysterectomy. Bladder + ureter check each surgery!

★ PNC PEARL: Squeeze, press, fill — then steel.

► Balloon drain 600 ml/h + unstable. Next?

Failed tamponade — OT surgery + blood + ICU.

FETAL DISTRESS / NON-REASSURING STATUS

Lates + no variability + thick meconium + scalp acid = DELIVER! Position + O2 + fluids + oxytocin OFF + full bladder empty + senior + OT — bradycardia <100 >3 min = crash section (consent + blood)!

Lates + flat
+ thick = out

Oxytocin OFF
+ position

<100 >3 min
= crash

Σ SIGN – ACT

Sign	Act
Late/variable + poor variability	Resuscitate + prepare OT
Thick meconium + abnormal	Expedite; NRP team ready
Brady <100	Crash CS + blood + NICU + peds
Cord/APH cause	Fix cause + deliver together

⚠ FATAL TRAP

Vacuum/forceps by junior on high station + 'one more push hour' with flat trace asphyxiates! Station + pelvis + urgency pick mode (assisted low + favorable vs CS) — decision-to-delivery <30 min.

★ NORCET HIT

HIT: Intrauterine resuscitation bundle + SBAR + consent + left + O2 + stop oxytocin + treat hypotension cause. Document trace + actions + times!

★ PNC PEARL: Flat trace + thick stain = wheels rolling.

► Flat + lates + thick + 9 cm primi. Mode?

Non-reassuring — CS (assisted only if low + skilled).

RUPTURED UTERUS

Prior CS + obstructed + oxytocin + Bandl + hematuria + shock + lost station = RUPTURE! Stop oxytocin + fluids + blood + crash OT (repair vs hysterectomy) + bladder check + NICU!

Scar + obstruct + oxytocin = risk	Bandl + hematuria + shock	Stop drug + crash OT
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⚠ SIGN — ACT

Sign	Act
Tearing pain + shock	Stop oxytocin + fluids + blood
Lost station + hematuria	Bladder + rupture confirm + OT
Fetal brady/distress	Crash section + NICU
Post	Repair/hysterectomy + bladder + ICU

⚠ FATAL TRAP

More oxytocin + vaginal 'trial' after Bandl + hematuria + shock completes the tear + kills both! Rupture = abdominal + blood + bladder/ureter check + consent + HDU + next-CS counsel.

★ NORCET HIT

HIT: Dehiscence (silent window) vs rupture (catastrophic) — scar pain + FHR changes warn! VBAC needs 1 prior + OT + blood + consent + partograph.

★ PNC PEARL: Scar + Bandl + blood-urine = blade.

► VBAC + Bandl + hematuria + shock. Act trio?

Stop oxytocin + blood + crash OT + bladder check.

Gush + something per vaginum + FHR crash = PROLAPSE! Knee-chest/Trendelenburg + gloved lift + O2 + crash OT — hand STAYS till baby out! Risks: malpresentation + poly + prematurity + high head + ARM!

Gush + mass +
crash = prolapse

Knee-chest +
lift + hold

Hand stays till
baby out

▲ STEP — DO

Step	Do
Position	Knee-chest/Trendelenburg + lift presenting part
Oxygen + fluids	Mother O2 + IV + stop oxytocin
Bladder	Fill 500 ml to lift head (if protocol)
Birth	Crash CS; vaginal only if imminent + skilled

⚠ FATAL TRAP

Pushing cord back + 'wait for full' + walking her to OT compresses + spasms it dead! Minimal handle + warm saline wrap + continuous lift + FHR + OT clock — lifting saves, touching kills.

★ NORCET HIT

HIT: Occult (alongside) vs overt (beyond) — FHR + VE diagnose. Never cut/clamp outside! Document times + FHR + mode + Apgar + debrief.

★ PNC PEARL: Lift, don't touch; rush, don't walk.

► Cord at vulva + FHR 80 + 6 cm. Bundle?

Knee-chest + lift + O2 + crash CS + NICU.

Head out + turtle + no restitution = DYSTOCIA! McRoberts + suprapubic + Woods/screw + all-fours — NO fundal push, NO traction! Call + clock + NICU + brachial/HIE watch + honest debrief!

**Turtle + stuck
= dystocia**

**McRoberts + supra
first**

**NO push/pull
ever!**

◆ STEP — DO

Step	Do
McRoberts	Hyperflex hips + suprapubic pressure
Internal	Woods/screw + posterior arm delivery
All-fours	Gaskin + gravity + space
After	Brachial check + HIE watch + debrief

⚠ FATAL TRAP

Fundal pressure + head yanking + panic cut without space avulses plexus + asphyxiates! Calm + roles + maneuvers + time-box each (30–60 s) + document + peds + physio.

★ NORCET HIT

HIT: Predictors: GDM + macrosomia + prior dystocia + prolonged 2nd — plan CS threshold. Erb's (C5–6 waiter's tip) vs Klumpke (C8–T1 claw + Horner)!

★ PNC PEARL: Legs up, hand above, patience between.

► Turtle + no restitution 60 s. First 2?

McRoberts + suprapubic + call + clock + NICU.

INVERSION + VASA PREVIA

Fundus at vulva + shock + bleed = INVERSION — placenta stays, push back + relax + OT! Vasa (ROM + fetal bleed + brady) = crash CS + neonatal blood — pale floppy baby transfused fast!

Inversion: push back + relax	Vasa: fetal bleed + crash	Shock out of proportion!
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⚠ EVENT — ACT

Event	Act
Inversion	Replace + tocolysis + OT + blood + antibiotics
Vasa bleed	Crash CS + cord gases + neonatal transfusion
Both	Shock + DIC labs + ICU + debrief
Prevent	No cord pull pre-signs; vasa screen IVF/twins

⚠ FATAL TRAP

Pulling placenta off inverted uterus + 'push harder' + delayed OT = neurogenic + hemorrhagic shock + death! Johnson maneuver + relaxant + anesthesia + blood + HDU + next-pregnancy counsel.

★ NORCET HIT

HIT: Inversion degrees (dimple → vulva → outside).
Vasa Apt pink = fetal — NICU before incision!

★ PNC PEARL: Replace first, remove never (till OT).

► Fundus at introitus + shock + attached placenta. Act?

Replace + relax + OT + blood — placenta stays!

Collapse + DIC + breathlessness ± seizures around labour/ARM = AFE! O2 + fluids + blood + ICU + deliver fast — never give up (ROSC happens!) — shock always needs CAUSE: bleed vs clot vs heart vs sepsis!

Collapse + DIC + dyspnea = AFE	O2 + blood + deliver + ICU	Shock = cause-hunt always
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Σ SHOCK — CLUE — ACT

Shock	Clue	Act
Hemorrhagic	Bleed + cold + tachy	Blood + TXA + cause + OT
AFE	Collapse + DIC + hypoxia	O2 + blood + deliver + ICU
Cardiogenic	Wet + JVP up	Sit + O2 + diuretics + team
Septic	Fever + warm then cold	Cultures + antibiotics + source

⚠ FATAL TRAP

Heparin for AFE-DIC + fluids-only for cardiogenic + 'stabilize then shift' = multi-death! AFE = supportive + deliver + products (heparin HARMS!) + echo + debrief + registry.

★ NORCET HIT

HIT: AFE triad: hypoxia + hypotension + coagulopathy ± seizures. Left tilt + 100% O2 + massive protocol from minute one!

★ PNC PEARL: Collapse in labour: think amniotic first.

► ARM + sudden collapse + DIC + hypoxia. Name + bundle?

AFE — O2 + blood + deliver + ICU + no heparin.

>30 min or bleed = RETAINED! Signs + contraction + counter-pressure + Brandt-Andrews — accreta (prior CS + previa) NEVER forced; OT manual removal + blood + antibiotics + HDU!

30 min/bleed = retained	Signs + squeeze + counter	Accreta: NEVER force!
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▲ CAUSE — ACT

Cause	Act
Trapped (closed OS)	Relax + anesthesia + gentle removal
Adherent	Controlled + OT manual under anesthesia
Accreta spectrum	No pull — hysterectomy team + blood + ICU
Succenturiate lobe	Explore + remove bits + antibiotics

⚠ FATAL TRAP

Yanking cord + Crede without signs + unsterile manual scoop = inversion + sepsis! Asepsis + anesthesia + single confident exploration + oxytocin + TXA + count + HDU.

★ NORCET HIT

HIT: Hourglass ring needs relaxation + anesthesia. Post: Hb + Rh + BF + FP + debrief + next-accreta counsel!

★ PNC PEARL: Half hour or hemorrhage — hands in, lights on.

► 40 min + trapped + no bleed. Act?

OT manual removal + anesthesia + blood ready.

Inspect EVERY birth: vulva + vagina + cervix + perineum + rectum! Atonic bleeds dark-steady + boggy; TRAUMA bleeds bright + firm uterus — speculum + repair + rectal check before/after, senior for 3rd/4th!

Bright + firm = trauma	Dark + boggy = atony	Rectal check before + after
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★ SITE — REPAIR

Site	Repair
Vulval/vaginal tears	Layered + hemostasis + pack if ooze
Cervical tear	Speculum + ring forceps + stitch + senior
Perineal 3rd/4th	OT + layers + antibiotics + soft stool
Hematoma	Small watch; expanding = evacuate + drain

⚠ FATAL TRAP

Missing a high vaginal/cervical tear + 'massage harder' while trauma bleeds wastes blood + time! Firm uterus + flood = speculum + light + repair + TXA + count — atony and trauma COEXIST.

★ NORCET HIT

HIT: Vulval hematoma + retention + severe pain = expanding until proven — ice + catheter + OT. Post: Hb + sitz + stool + FP!

★ PNC PEARL: Firm flood? Find the tear.

► Firm uterus + steady bright bleed post-episiotomy. Act?

Trauma — speculum + repair + hemostasis + count.

PRIMARY PPH (ATONIC FOCUS)

4 Ts: Tone (70%) + Tissue + Trauma + Thrombin! Call + massage + empty bladder + oxytocin + TXA <3 h + warmth + blood + cause-hunt TOGETHER — weighed pads + shock index guide!

4 Ts: tone + tissue + trauma	TXA <3 h + blood	Weigh pads + shock index
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⚠ CAUSE — FIRST MOVES

Cause	First moves
Tone	Massage + void + oxytocin + ergometrine/miso
Tissue/trauma	Explore + repair + tamponade
Thrombin	Labs + FFP/cryo + TXA early
Shock	Warm + O2 + 2 IVs + blood + OT

⚠ FATAL TRAP

'Wait for the doctor' + unmeasured 'some clots' + cold + delay = death by delay! Bundle + roles + kit + weighed pads + 15-min vitals + TXA <3 h + move early (balloon/surgery).

★ NORCET HIT

HIT: NASG buys transfer time. B-Lynch → devascularization → hysterectomy ladder — consent + senior + blood bank + ICU!

★ PNC PEARL: Massage, measure, medicate — in minutes.

► 500 ml + atonic + trickle after AMTSL. Bundle?

Massage + bladder + 2nd uterotonic + TXA + blood + senior.

PPH RESCUE: BIMANUAL, AORTIC, BALLOON

Hands + pressure + balloon BEFORE blade! Bimanual (fist + abdominal hand) + aortic press + Bakri/condom tamponade + drainage-track + OT threshold — each step timed + documented!

Bimanual fist + guard	Aortic press bridge	Balloon + drain track
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◆ STEP — HOW

Step	How
Bimanual	Fist anterior fornix + abdominal fundal press
Aortic	Fist above umbilicus, press spine, femoral stop
Balloon	Bakri/condom + fill + drainage bag track
Move on	Surgery if >500 ml drain/h or unstable

⚠ FATAL TRAP

Prolonged aortic press + balloon without drainage-track hides ongoing bleed! Time-box each (15–30 min) + I/O + vitals + labs — move up, never camp on one step.

★ NORCET HIT

HIT: Ladder: drugs → tamponade → compression sutures → devascularization → hysterectomy. Bladder + ureter check each surgery!

★ PNC PEARL: Squeeze, press, fill — then steel.

► Balloon drain 600 ml/h + unstable. Next?

Failed tamponade — OT surgery + blood + ICU.

FETAL DISTRESS / NON-REASSURING STATUS

Lates + no variability + thick meconium = DELIVER! Position + O2 + fluids + oxytocin OFF + bladder empty + senior + OT — bradycardia <100 >3 min = crash section!

Lates + flat
+ thick = out

Oxytocin OFF
+ position

<100 >3 min
= crash

Σ SIGN — ACT

Sign	Act
Late/variable + poor variability	Resuscitate + prepare OT
Thick meconium + abnormal	Expedite; NRP team ready
Brady <100	Crash CS + blood + NICU
Cord/APH cause	Fix cause + deliver together

⚠ FATAL TRAP

Vacuum/forceps by junior on high station + 'one more push hour' with flat trace asphyxiates! Station + pelvis + urgency pick mode — decision-to-delivery <30 min.

★ NORCET HIT

HIT: Bundle + SBAR + consent + left + O2 + stop oxytocin + treat hypotension. Document trace + actions + times!

★ PNC PEARL: Flat trace + thick stain = wheels rolling.

► Flat + lates + thick + 9 cm primi. Mode?

Non-reassuring — CS (assisted only if low + skilled).

EPISIOTOMY: RESTRICTIVE USE

Restrictive mediolateral ONLY (fetal + inelastic + operative + shoulder space)! Local + 7–8 o'clock at crowning + layered repair + rectal check — routine cuts + midline = fistula + pain + suit!

Restrictive + mediolateral	Local + layers + rectal check	Routine = harm
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▲ STEP — DO

Step	Do
Indicate	Distress + inelastic + operative + shoulder space
Cut	Local + mediolateral 7–8 o'clock at crowning
Repair	Layers + hemostasis + rectal before/after
After	Ice + sitz + stool + pain + follow-up

⚠ FATAL TRAP

Midline + no anesthesia + skipped rectal exam = missed 3rd + fistula + agony! Indication + consent + local + senior for extensions + antibiotics + dyspareunia counsel.

★ NORCET HIT

HIT: Angle 40–60° protects sphincter. Bleeding angle vessel = figure-eight + hematoma watch. Repair infection? + antibiotics + senior!

★ PNC PEARL: Angle saves sphincters.

► Extension + rectal exposure found. Who + where?

Senior/doctor + OT + layers + antibiotics + soft stool.

Repair like anatomy reads: mucosa + muscle + skin, hemostasis + dead-space closed + rectal check BEFORE and AFTER! Good light + local + lithotomy + count + sitz + stool plan — gaping/pus = back to OT!

Mucosa-muscle-skin order	Rectal before + after	Gape/pus = back to OT
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◆ LAYER — STITCH

Layer	Stitch
Vaginal mucosa	Continuous locking + apex first
Muscle	Interrupted + dead-space closed
Skin	Subcuticular + align edges
Check	Rectal + vaginal + bleed + count

⚠ FATAL TRAP

Stitch through rectum + tight skin + no count + 'go home' = fistula + pain + retained pack! Rectal before/after, mucosa stitch removed + redone, pack count aloud + documented.

★ NORCET HIT

HIT: 2nd-degree by trained nurse-midwife; 3rd/4th = doctor OT. Ice + analgesia + sitz + stool + 6-w perineal + FP review!

★ PNC PEARL: Apex first, rectum checked twice.

► Mucosal stitch on rectal recheck. Act?

Remove + redo layer + recheck + document.

EPISIOTOMY + PERINEAL SUTURING

Restrictive mediolateral ONLY (fetal + inelastic + operative)! Local + 7–8 o'clock + layers + rectal check before/after — routine cuts + midline + missed 3rd = fistula + pain + suit!

Restrictive + mediolateral	Local + layers + rectal check	Routine = harm
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▲ STEP — DO

Step	Do
Indicate	Fetal distress + inelastic + operative + shoulder space
Cut	Local + mediolateral 7–8 o'clock at crowning
Repair	Layers + hemostasis + rectal before/after
After	Ice + sitz + stool + pain + dyspareunia counsel

⚠ FATAL TRAP

Midline + no anesthesia + skipped rectal exam + early sex = 3rd-degree missed + fistula + agony! Indication + consent + local + senior for extensions + antibiotics + follow-up + FP.

★ NORCET HIT

HIT: Angle 40–60° protects sphincter. Bleeding angle vessel = figure-eight + watch hematoma. Repair <6 h infected? + antibiotics + senior call!

★ PNC PEARL: Angle saves sphincters.

► Extension + rectal exposure found. Who + where?

Senior/doctor + OT + layers + antibiotics + soft stool.

FORCEPS VS VACUUM

Low + OA + engaged + full + empty bladder + consent + OT backup = assisted;
HIGH/transverse/deflexed/CPD = CS! Forceps pulls + rotates (facial marks, nerve watch);
vacuum sucks + pops (cephalohematoma, no prematurity <34)!

Low + OA +
engaged only

Forceps: pull +
nerve watch

Vacuum: pop-offs
3 = stop

★ TOOL – RULE

Tool	Rule
Forceps	Low/OA + traction + rotation + facial/nerve check
Vacuum	34+ wks + 3 pulls/pops = stop + CS
Both need	Full + station + pelvis + consent + NICU
Never	High station, CPD, deflexion, coagulopathy

⚠ FATAL TRAP

Vacuum on <34 wks + 5 pop-offs + fundal push + high station bleeds brain + fails! Time-box + pop-count + FHR + maternal effort + senior + CS threshold — failed = OT, never home-trial.

★ NORCET HIT

HIT: Chignon (vacuum bump) fades; cephalohematoma stays + jaundice watch; forceps marks + facial palsy watch. Episiotomy only if space needed!

★ PNC PEARL: Three pops, then steel (CS).

► Vacuum 3 pop-offs + no descent + FHR dip. Next?

Failed — stop + crash CS + NICU + document.

Turn breech **OUTSIDE** at 36–37 wks: tocolysis + FHR + flexed + adequate fluid + OT + blood + anti-D ready! No-go: ROM, bleeding, scar-risk, anomaly, twins — bradycardia >3 min = crash CS!

36–37 wks + tocolysis	FHR + OT + blood ready	Brady + ROM/ bleed = stop
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◆ STEP — DO

Step	Do
Select	Breech + flexed + fluid + no scar-risk/bleed
Prep	Tocolysis + FHR + consent + OT + anti-D
Turn	Gentle + FHR breaks + stop on pain/brady
After	FHR + CTG + anti-D + birth plan + CS backup

⚠ FATAL TRAP

Forceful turn + no FHR + abruption ignored + 'try once more' abruptly + knots cord! Stop on pain/bradycardia/bleed; abruption/DIC labs + OT ready — success still needs labour vigilance.

★ NORCET HIT

HIT: Success ~50% + reversion possible — rescan on admission! Transverse + stable + skilled = ECV option; labour transverse = CS.

★ PNC PEARL: Gentle hands, watched heart, open OT.

► ECV + persistent bradycardia 4 min. Act?

Stop + left + O2 + crash CS + NICU.

INDUCTION OF LABOUR (PRINCIPLES + METHODS)

Induce with INDICATION + Bishop + consent + bed + FHR! Foley/Sweep/ARM + oxytocin/miso per Bishop + hyperstimulation watch (stop + lateral + O2) — failed = CS, never endless titration!

Indication + Bishop + bed	Foley/sweep/ARM + drugs	Hyperstim = stop + lateral
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Σ METHOD — RULE

Method	Rule
Sweep/Foley	Unfavourable + intact + GBS screen; traction-free
ARM	Engaged + no vasa/HIV-active + FHR + cord check
Oxytocin	Low-dose titration + 1:1 + hyperstim protocol
Misoprostol	Dose/route per protocol + scar caution + watch

⚠ FATAL TRAP

Miso + scar + hyperstimulation + 'one more dose' ruptures + asphyxiates! Scar = Foley/mechanical preferred + low threshold CS; ARM + polyhydramnios/vissa = controlled + cord check + OT ready.

★ NORCET HIT

HIT: Bishop ≥6 favourable. Failed induction = CS after adequate trial + senior + consent + debrief. Outpatient sweep + dates + GBS + transport counsel!

★ PNC PEARL: Ripe cervix, watched uterus, open OT.

► Miso + tetanic + FHR lates + scar. Act trio?

Stop + lateral + O2 + OT + rupture watch.

CS saves when vaginal threatens: CPD/obstruction/previa/vasa/distress/rupture/transverse + maternal request counselled! Prep = NPO + consent + blood + antibiotics <60 min + catheter + count + NICU + VTE plan!

Threat = section	NPO + blood + antibiotics	Count + NICU + VTE plan
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★ STEP – DO

Step	Do
Indicate	CPD/obstruct/previa/vasa/distress/rupture + counsel
Prep	NPO + consent + blood + antibiotics + catheter
Safety	Surgical count + NICU + placenta/accreta team
After	ERAS + analgesia + VTE + wound + BF + debrief

⚠ FATAL TRAP

'CS on demand' without risk counsel + VBAC without OT/blood + classical-scar vaginal labour = rupture + regret + suit! Indication + consent + scar-type record + next-pregnancy plan documented.

★ NORCET HIT

HIT: Category-1 (<30 min) vs elective lists. Accreta + previa = midline + blood + hysterectomy consent + ICU. ERAS: chew + mobilize + catheter out early!

★ PNC PEARL: Right cut, right cause, recorded consent.

► Prev CS + previa + accreta signs. Prep + team?

Elective CS + midline + blood + hysterectomy consent + ICU.

POST-CS CARE + OBSTETRIC ANAESTHESIA

ERAS after steel: analgesia + antiemetic + early feed/mobilize + catheter-out + wound watch + VTE + BF + debrief! Spinal headache + high block + hypotension + aspiration = tilt + O2 + vasopressor + airway ready!

**ERAS: feed +
walk + out**

**Spinal: tilt +
O2 + pressor**

**Headache + back
watch**

▲ ITEM — RULE

Item	Rule
ERAS	Multimodal analgesia + chew + walk + catheter <24 h
Wound/VTE	Inspect + stockings + LMW heparin risk-based
Spinal/epidural	Tilt + fluids + pressor; PDPH = fluids + caffeine + patch
GA risks	Aspiration + difficult airway + baby depression

⚠ FATAL TRAP

Flat + NPO + catheter for days + no VTE plan + missed wound infection = clot + sepsis + readmit! Walk day-0 + chest physio + wound photo + fever/bleed card + 6-w + FP + next-CS date.

★ NORCET HIT

HIT: High spinal (hands numb + dyspnea) = airway + tilt + pressor + OT pause. Aspiration prophylaxis: antacid + RSI + cricoid + suction!

★ PNC PEARL: Walk early, clot never, wound watched.

► Post-spinal severe positional headache. Act?

PDPH — fluids + caffeine + analgesia + blood-patch plan.

DRUGS: ABNORMAL LABOUR + EMERGENCIES

Pocket the peripartum pharmacy: oxytocin/ergometrine/miso (tone) + TXA <3 h + MgSO4 + nifedipine + steroids + antibiotics + anti-D + Rh + insulin + blood! Dose + dilution + contraindication + antidote — every label double-checked!

Tone + TXA <3 h	Mg + steroids windows	Double-check every label
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Σ DRUG — USE / WATCH

Drug	Use / watch
Oxytocin/ergometrine/miso	Tone; ergot HTN caution; miso scar caution
TXA	PPH <3 h 1 g IV; clot history screen
MgSO4/nifedipine	Seizure/neuroprotection; BP + DTR watch
Steroids/antibiotics/anti-D	Windows + GBS + Rh + allergy band

⚠ FATAL TRAP

Ergometrine in HTN + oxytocin bolus in shock + TXA after 3 h + 'verbal milligrams' = stroke + collapse + miss! Read-back + dilution + slow IV + vitals + antidote (calcium/fluids/pressors) bedside.

★ NORCET HIT

HIT: High-alert stickers + separate storage + expiry + cold chain. Anaphylaxis kit with EVERY biologic/iron/antibiotic first dose!

★ PNC PEARL: Right drug, right dilution, read back.

► PPH 2.5 h + shock + no TXA yet. Give?

Yes 1 g IV now (<3 h) + blood + cause + OT.

OBSTETRIC PROCEDURES: NURSING OVERVIEW

Every procedure = indication + consent + checklist + count + sample + handover! ARM, ECV, assisted, CS, MVA, D&C;, sterilization, cerclage — prep + position + parts + pathology + plan, no exceptions!

Indication + consent + list	Count + sample + label	Handover + debrief all
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◆ STEP — DO

Step	Do
Before	Indication + consent + NPO + blood + antibiotics
During	Position + monitoring + count + samples labelled
After	Vitals + bleeding + void + pain + BF + VTE
Records	Notes + partograph + register + MCTS + debrief

⚠ FATAL TRAP

Missing swab count + unlabeled products + 'routine' discharge without void/bleed check = retained + sepsis + readmit! Count aloud + label bedside + void + walk + card + dated return.

★ NORCET HIT

HIT: Cerclage (history-indicated/prophylactic) + bed + infection watch + removal date. MVA <12 wks + D&C; consent + Rh + antibiotics + FP same visit!

★ PNC PEARL: Count, label, void, walk — discharge's four gates.

► Post-D&C; + no void 6 h + distended. Act?

Retention — encourage + catheter + measure + cause.

POSTNATAL DEVIATION CHECK

Normal ends where RED begins! Fever + foul + flood + calf pain + chest pain + blues >2 wks + baby flags (poor suck, yellow palms, fever) = ADMIT, never 'next visit'! Dyad checked together, every time!

Fever/foul/flood = admit	Calf/chest/blues = red	Dyad together every check
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★ FLAG — ACT

Flag	Act
Mother vitals	Fever + tachy + hypotension = sepsis bundle
Bleed/discharge	Flood/foul = PPH/endometritis workup
Legs/chest/mood	DVT/PE/psychosis = emergency lane
Baby	Poor suck + yellow palms/soles + fever = NICU

⚠ FATAL TRAP

'Milk fever + tiredness' label on Day-5 sepsis + retained bits + PE + psychosis kills TWO! Vitals + uterus + perineum + legs + mood + baby head-to-toe — red = admit + senior, never reassure-and-discharge.

★ NORCET HIT

HIT: MEOWS charts puerperium too. Home-visit trio (day 3/7/42) catches 80% deviations — ASHA + ANM + MO chain with referral slips!

★ PNC PEARL: Red never waits for morning.

► Day-5 fever + flood + tachy. Lane?

Sepsis + PPH lane — admit + bundle + senior.

Fever ≥ 38 + tachy + tender boggy uterus + foul lochia = SEPSIS! Cultures + broad IV antibiotics <1 h + fluids + empty uterus (retained?) + VTE cover — home-antipyretic + ‘milk fever’ thinking kills!

**Fever + tender
+ foul = sepsis**

**Cultures + IV
<1 hour**

**Empty uterus
+ fluids**

⚠ STEP — SEPSIS SIX

Step	Do
Cultures	Blood + high vaginal + urine BEFORE antibiotics
Antibiotics	Broad IV <1 h + source dose
Fluids + O2	30 ml/kg + lactate + urine track
Source	Retained? explore + remove + pack check
Support	VTE + glucose + nutrition + HDU

⚠ FATAL TRAP

Vaginal exam + packing + ‘observe till morning’ in septic shock seeds + delays! Sterile + minimal + early + HDU + repeat lactate — each hour of antibiotic delay raises death.

★ NORCET HIT

HIT: Risk stack: PROM + prolonged labour + VE + CS + anemia + poor hygiene. Tetanus + gas gangrene (foul + crepitus + shock) = OT + ICU + report!

★ PNC PEARL: Hour-one antibiotics beat hour-ten apologies.

► Day-4 fever + foul + tender uterus + tachy. Bundle?

Cultures + broad IV <1 h + fluids + retained hunt.

Void <6 h or intervene! Retention (atonic + edema + epi pain) → timed + privacy + warm water → in-out/catheter + record. Dysuria + fever = UTI culture + treat; leaking = fistula hunt, never pads-forever!

Void <6 h or act	Dysuria + fever = culture	Leaking = fistula hunt
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▲ ISSUE – ACT

Issue	Act
Retention	Encourage + privacy + warm water → catheter
UTI/cystitis	Culture + antibiotics + fluids + retest
Incontinence-stress	Pelvic floor + review; fistula if continuous
Fistula leak	Dye test + referral + catheter + counsel

⚠ FATAL TRAP

Leaving catheter weeks + 'pads for leak' + missed VVF after obstructed labour destroys dignity! Timed removal + void trial + residual check + fistula centre link + psych + FP bridge.

★ NORCET HIT

HIT: Postpartum diuresis is normal (edema mobilizes); oliguria + HTN + protein = PE watch continues! Bladder scan residual >150 = retention protocol.

★ PNC PEARL: Six hours, one void, zero leaks ignored.

► Day-2 continuous leak after obstructed labour. Act?

Suspect VVF — dye test + referral + catheter + counsel.

Bleed AFTER 24 h–2 wks = retained + infection until proven! Foul + tender + boggy + fever + open OS = antibiotics + exploration + oxytocics + blood + histology — choriocarcinoma rare but real on recurrence!

24 h–2 wks +
foul = retained

Explore +
antibiotics

Recurrence =
histology!

◆ CAUSE – ACT

Cause	Act
Retained bits	USG + exploration + antibiotics + oxytocics
Endometritis	Broad IV + retained hunt + HDU
GTN/choriocarcinoma	hCG + histology + chemo-curable referral
Coagulopathy	Labs + products + senior + OT

⚠ FATAL TRAP

Curettage + discharge without antibiotics + histology + hCG follow-up = sepsis + missed GTN! Gentle + sterile + sample-labelled + Rh + FP + dated review — perforation watch (soft infected uterus)!

★ NORCET HIT

HIT: Secondary peaks day 7–14. Subinvolution (boggy + heavy) rides along — ergometrine + antibiotics + massage + void. hCG follow-up after molar pregnancy history!

★ PNC PEARL: Late bleed = leftovers or infection — prove which.

► Day-10 flood + foul + boggy + fever. Bundle?

USG + exploration + broad IV + oxytocics + blood.

VULVAL/VAGINAL HEMATOMA

Severe pain + retention + tense swelling + falling Hb = EXPANDING until proven! Small + stable watches (ice + catheter + Hb); expanding/pulse/BP fall = evacuate + drain + pack + blood + OT!

Pain + retention + swell = watch	Expanding/pulse = evacuate	Ice + catheter + Hb track
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★ SIZE — ACT

Size	Act
Small + stable	Ice + analgesia + catheter + Hb + watch
Expanding	Evacuate + ligate + drain + pack
Shock	Blood + OT + ICU + coag labs
Post	Sitz + stool + antibiotics + FP + review

⚠ FATAL TRAP

'Perineal pain is normal' + no look + no vitals hides liters in tissues! Undress + inspect + vitals + Hb + bladder + senior at FIRST severe pain/retention — concealed bleeds kill quietly.

★ NORCET HIT

HIT: Broad-ligament (high) hematoma = pain + shock + NO vulval swell — USG + OT + blood! Post-episiotomy oozers = figure-eight + pressure + watch.

★ PNC PEARL: Pain + retention + swelling = look NOW.

► Severe perineal pain + can't void + tense swelling. Act?

Expanding — ice + catheter + Hb + OT evacuation ready.

ENGORGEMENT, BLOCKED DUCT & OTHER BF ISSUES

Feed through fullness! Frequent + correct latch + warm-pre/cold-post + hand-express + different holds unblock; fever + wedge-red = mastitis line crossed — bind + stop = abscess highway!

Feed q2 h +
latch fix

Warm pre/cold
post + express

Fever + wedge
= mastitis!

▲ ISSUE — FIX

Issue	Fix
Engorgement	Frequent + latch + express + cabbage/cold
Blocked duct	Same + lump-toward-nipple massage + heat
Sore/flat nipple	Deep latch + holds + lanolin + air
Low supply fear	Weigh + pees + cluster-feed counsel

⚠ FATAL TRAP

Binding + 'rest the breast' + top-ups for engorgement = mastitis + supply crash! Empty often + correctly + support + sleep-shift + weight proof — supplement only with indication + plan.

★ NORCET HIT

HIT: inverted/flat = syringe + shield + patience. Thrush (shooting + white) = both treated + hygiene. Work-return = express + store + cup-feed plan!

★ PNC PEARL: Empty + often + correctly — the unblock trio.

► Lump + tender + no fever + feeding well. Act?

Blocked duct — feed + massage + heat + watch 24 h.

Wedge-red + fever + ache = MASTITIS: continue feeding + antibiotics + support + rest! Fluctuant + throbbing + pus = ABSCESS: stop that side + aspirate/incise + culture + admit — delay bursts + fistulizes!

**Wedge + fever
= mastitis**

**Fluctuant + pus
= abscess**

**Feed mastitis;
pause abscess side**

⚠ STAGE — ACT

Stage	Act
Mastitis	Feed on + antibiotics + support + rest + fluids
Abscess	Stop side + aspirate/I&D; + culture + admit
Sepsis	IV + HDU + glucose + baby EBM plan
Heal	Wound + BF re-establish + FP + review

⚠ FATAL TRAP

Hot fomentation + squeezing + 'milk injection' quackery on abscess + stopping ALL feeds dehydrates baby + bursts breast! Sterile + surgical + feed healthy side + EBM + senior + follow-up.

★ NORCET HIT

HIT: Cracked entry + missed feeds + tight bras stack risk. Milk culture guides antibiotics; MRSA cover per protocol. Pain + red flag card at discharge!

★ PNC PEARL: Mastitis feeds, abscess rests (that side).

► Fluctuant + fever + pus on express. Act?

Abscess — aspirate/I&D; + culture + admit + EBM plan.

Pregnancy clots 5x! Unilateral pain + swelling + warmth (no massage!) = Doppler + anticoag + stockings + walk rest of body. Chest pain + breathless = PE — O2 + heparin + CT + ICU, no delay!

No massage ever!	Doppler + anticoag	Chest pain = PE lane
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◆ CLUE — ACT

Clue	Act
Calf pain + swell	No massage + Doppler + LMW heparin
Superficial cord	Warm + NSAID + watch + walk
PE signs	O2 + heparin + CT + ICU + monitor
Prevent	Early walk + fluids + stockings + risk heparin

⚠ FATAL TRAP

Massage + hot pack + 'walk it off' for DVT + ignoring postpartum chest pain as 'gas' embolizes fatally!
Risk-score + prophylaxis + symptoms card + 6-w follow-up + next-pregnancy plan.

★ NORCET HIT

HIT: CS + immobility + obesity + age + prior clot stack risk. Warfarin postpartum ok (not pregnant!) + bridging + INR + bleed watch. Homan's unreliable — Doppler rules!

★ PNC PEARL: Clot: don't touch, do Doppler.

► Day-7 calf swell + pain + Homan +. Act?

No massage + Doppler + anticoag + stockings.

Boggy + heavy + backache + foul = uterus stuck! Retained + infection + full bladder + overwork stack — massage + void + ergometrine + antibiotics + rest + recheck + histology if bits!

Boggy + heavy = stuck	Massage + void + ergot	Bits/histology if retained
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Σ CAUSE — FIX

Cause	Fix
Retained bits	USG + explore + histology + antibiotics
Infection	Broad IV + HDU + fluids
Full bladder/overwork	Void + rest + binder + help
Fibroid/anomaly	Scan + senior + plan

⚠ FATAL TRAP

'Heavy is normal for 2 weeks' + no exam + no Hb = anemia + sepsis + readmit! Fundus + lochia + vitals + Hb + USG + dated review — secondary PPH protocol if flooding.

★ NORCET HIT

HIT: Involution 1 cm/day yardstick. Prolonged rubra + clots + backache = textbook stem. Breastfeeding + early walk HELP involution!

★ PNC PEARL: Boggy + bleed + backache = bits or bugs.

► Day-9 boggy + heavy + foul. Bundle?

USG + explore + antibiotics + ergometrine + Hb.

Continuous leak + excoriation after obstructed labour/CS = FISTULA till proven! Dye test + referral + catheter + skin + psych + FP bridge — repair at fistula centre + reintegration, never shame!

Continuous leak = fistula	Dye + refer + catheter	Repair + dignity + FP
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▲ TYPE — CLUE — ACT

Type	Clue	Act
VVF	Urine leak continuous	Dye + catheter + referral
RVF	Flatus/stool per vaginum	Bowel prep + referral
Both	Obstructed labour/CS history	Centre + psych + FP
Post	3-mo catheter + continence + review	Reintegrate + counsel

⚠ FATAL TRAP

Pads-forever + divorce-shame + quack cautery + early repair failure = lifelong leak! Small + early + catheter heals some; surgical timing + expert + tissue + follow-up + next-CS plan.

★ NORCET HIT

HIT: Prevention = partograph + CPD + no prolonged obstruct + catheter care. Skin zinc + fluids + support group + livelihood rebuild dignity!

★ PNC PEARL: Leak located, lady linked, life rebuilt.

► Continuous urine leak post-obstructed labour. Act trio?

Dye test + catheter + fistula-centre referral.

Beyond blues (>2 wks): hopeless + detached + no joy + appetite/sleep crash = DEPRESSION!
EPDS ≥13 + safety + psych + family shifts + meds where indicated — bonding + baby care suffer together, treat together!

Blues <2 wks fades	Depression: hopeless + detached	EPDS + safety + psych
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★ FEATURE — SIGN — ACT

Feature	Sign	Act
Mood	Hopeless + guilt + no joy	EPDS + listen + validate
Bonding	Detached + care lapses	Support + supervised care
Body	Sleep/appetite crash	Rest shifts + nutrition
Danger	Harm thoughts	Never alone + emergency

⚠ FATAL TRAP

'Good mothers don't feel this' + hiding pills + leaving alone with baby = tragedy pair! Normalize + screen + involve partner + 4-h sleep block + dated review — meds compatible with BF exist!

★ NORCET HIT

HIT: Thyroid + anemia mimic — labs too. Partner depression screens as well! Mother-baby unit admission beats separation where available.

★ PNC PEARL: Ask, don't assume — every postnatal contact.

► 3 wks hopeless + EPDS 16 + bonded poorly. Act?

Depression — psych + safety + family + review.

PSYCHOSIS + DRUGS + REFERRAL CHAIN

Confusion + delusions + sleepless mania + harm = PSYCHOSIS emergency — admit (mother-baby unit!) + antipsychotic + never alone! Drugs need indication + BF check + closed-loop referral + counter-slip + ASHA follow-up!

**Confusion/delusion
= admit**

**Never alone
+ safety**

**Slip + counter-slip
+ ASHA track**

⚠ ITEM — RULE

Item	Rule
Psychosis	Admit + meds + safety + family + ECT? per psych
Drugs + BF	Indication + infant risk + monitor + document
Referral	Slip + records + blood + call + transport
Loop	Counter-slip + ASHA + dated review + FP

⚠ FATAL TRAP

Sedate-and-send-home + 'evil eye' rituals + stopping all meds 'for milk' = infanticide/suicide + relapse! Medical + safety + rituals-respectful + follow-up + next-pregnancy preconception plan.

★ NORCET HIT

HIT: Bipolar/previous psychosis = highest risk — antenatal flag + birth plan + sleep protection + family psychoeducation from pregnancy itself!

★ PNC PEARL: Psychosis admits; safety never sleeps.

► Day-6 confused + delusions + won't sleep. Act?

Emergency — admit + safety + psych + family + meds.

HIGH-RISK NEWBORN + CARE LEVELS

Sort by RISK at birth: weight + gestation + asphyxia + anomaly + maternal flags! NBCC (birth) → NBSU (sick-stable) → SNCU (intensive) — warm + oxygen + glucose + feed + referral slip + call-ahead + transport WITH care!

Sort at birth by risk	NBCC→NBSU→ SNCU ladder	Refer WITH warmth + O2
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★ LEVEL — KEEPS

Level	Keeps
NBCC (birth unit)	Warm + resuscitate + stabilize + refer
NBSU (step-down)	O2 + feeds + jaundice + sepsis-watch
SNCU (intensive)	Vent/CPAP + lines + exchange + surgery link
Transport	Warm + O2 + glucose + feed + slip + call

⚠ FATAL TRAP

'Small but fine' + home discharge + cold open-auto transport = dead on arrival! Score + warm + glucose + O2 + feed + KMC + referral + family + money + feedback — SNCU bed confirmed before wheels!

★ NORCET HIT

HIT: High-risk flags: <2.5/>4 kg, <37/>42 wks, asphyxia, RD, jaundice <24 h, anomaly, diabetic/septic mother. FBNC + HBNC + JSSK cashless chain!

★ PNC PEARL: Right baby, right bed, warm wheels.

► 1.8 kg + grunting + cold. Level + move?

SNCU — warm + O2 + glucose + call + transport.

Born early = lungs + brain + gut + eyes immature! Warm + feed (EBM!) + infection wall + ROP/ hearing screens + KMC + weight + apnea watch — steroids helped before, surfactant + CPAP after!

**Warm + EBM
+ wall**

**KMC + weight
+ apnea watch**

**ROP + hearing
screens**

▲ SYSTEM — RISK — GUARD

System	Risk	Guard
Lungs	RDS + apnea	CPAP + caffeine + O2 blenders
Gut	NEC + feeds	EBM + slow + sterile + watch
Eyes/ears	ROP + deafness	O2 titrated + screens
Brain/bone	Bleed + soft	Gentle + Vit D + iron

⚠ FATAL TRAP

100% O2 + cold + formula + 'big feeds grow fast' = ROP + NEC + crash! Blended O2 + EBM + KMC + handwash + minimal handling + apnea monitor + parents-partners in care.

★ NORCET HIT

HIT: Kangaroo + EBM + early CPAP = survival trio.
Discharge: weight + feeds + warmth + danger + ROP/hearing dates + HBNC visits!

★ PNC PEARL: Small steps, warm chest, mother's milk.

► Preterm + apnea + desats. Guard trio?

CPAP + caffeine + KMC + O2 blended + monitor.

Weight <2.5 kg OR small-for-dates = feed + warm + watch extra! Postmature (>42 wks): dry + meconium + asphyxia + PPHN risk — dates + Doppler + deliver, never 'wait for pains' past term!

<2.5 kg = extra care	>42 wks = deliver, not wait	Feed + warm + watch trio
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◆ TYPE — PLAN

Type	Plan
Preterm LBW	Warm + feeds + O2 + screens + KMC
Term SGA	Glucose + feeds + anomaly/Doppler review
Postmature	Deliver + meconium + asphyxia + PPHN watch
All	Weight + danger + follow-up + card

⚠ FATAL TRAP

Fat-chart reassurance + home care for SGA + postdates wait + 'big mothers have big babies' = stillbirth + MAS! Growth + Doppler + dates + kick counts + delivery thresholds + consent.

★ NORCET HIT

HIT: Ponderal index + HC/AC + length tell symmetry.
Postmature: vernix-gone + long nails + dry + meconium + non-stress + amniotic check!

★ PNC PEARL: Dates + Doppler + weight — deliver on data.

► 42+ wks + meconium + non-reassuring. Act?

Deliver now + MAS + PPHN + NICU ready.

Chest-to-chest + cap + feed-access + back-support = warmer + milk + bond + brain! Stable <2.5 kg + any willing chest (mother/father!) + hours/day + weight + discharge when feeding + warm + gaining!

Chest + cap + feeds	Any chest works + hours/day	Weight + warm + gaining out
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★ STEP – DO

Step	Do
Position	Upright chest + cap + gown + airway open
Feed	EBM + access + q2 h + weight
Hours	As many as possible + night + father shifts
Out	Feeding + warm + gaining + card + dates

⚠ FATAL TRAP

KMC + covered nose + sleepy-unwatched + 'warmer is enough' = apnea + fall + cold! Airway visible + back tied + monitor + family roster + danger demo + chair + footstool + fluids for mother.

★ NORCET HIT

HIT: KMC cuts sepsis + hypo + death — evidence-strong! Twins + post-CS + working mother = roster + express + cup-feed + HBNC support.

★ PNC PEARL: Chest outranks machine for the stable small.

► 1.9 kg stable + willing mother. Prescribe?

KMC hours + EBM q2 h + weight + monitor + card.

**No cry + limp + blue + slow heart = NRP NOW:
warm-dry-position-suction-PPV-chest-compressions-drugs! HIE stages (alert/lethargic/coma) +
cooling + seizures + organ watch — every minute unwatched injures brain!**

Dry-position- PPV-compress	HIE alert/lethargic /coma	Cool + seize- watch + organs
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⚠ STEP — NRP

Step	NRP
Warm-dry-stim	60 s + position + clear if blocked
PPV	Apneic/gasping/HR<100 + O2 blender
Compressions	HR<60 + drugs + lines + NICU
HIE care	Cooling + glucose + seizures + multi-organ

⚠ FATAL TRAP

Slapping + hanging upside-down + mouth-suction +
'wait for cry' + cold table asphyxiates + aspirates! Clock
+ roles + equipment-checked + warmth +
documentation + debrief + follow-up (CP risk).

★ NORCET HIT

HIT: Apgar guides + cord gases prove. Meconium
non-vigorous: OLD suction-everyone is OUT —
PPV-first per current NRP! Seizure + tone + suck stage
HIE.

★ PNC PEARL: Air in sixty, brain saved for life.

► Apneic + HR 80 after stim. Next?

PPV + O2 + HR track + NRP team + clock.

NEONATAL SEPSIS + UNIT INFECTION WALL

Poor suck + lethargy + temp swing + grunt + sclerema = SEPSIS till proven! Cultures + ampi-genta <1 h + fluids + glucose + O2 + lines + handwash-wall + cohort + rational antibiotics — meningitis + shock hunted together!

**Poor suck/lethargy
= sepsis**

**Cultures + drugs
<1 hour**

**Wall: hands +
cohort + rational**

▲ STEP — DO

Step	Do
Spot	Danger + vitals + glucose + screen
Strike	Cultures + ampi-genta <1 h + fluids
Support	O2 + warmth + feeds/NPO + lines + HDU
Wall	Hands + linen + cohort + de-escalate

⚠ FATAL TRAP

Bottle + over-handling + 'cover all with higher antibiotics' + no cultures breeds resistance + misses meningitis! LP where indicated + stop/review 48–72 h + EBM + KMC + HBNC watch.

★ NORCET HIT

HIT: Early (<72 h, maternal) vs late (unit/hands) — GBS + E. coli + Kleb patterns + CRP/procal + I/T ratio guide. Sclerema + shock + DIC = ICU + products!

★ PNC PEARL: Culture first, cocktail fast, narrow soon.

► Lethargy + poor suck + fever + grunt. Bundle?

Cultures + ampi-genta <1 h + fluids + HDU.

Cold kills quietly: pink-warm (36.5–37.5) → cold stress → moderate → severe (<32)! Warm chain + cap + skin + no bath + KMC + rewarm slow + glucose check — cold + hypo + hypoxia travel together!

36.5–37.5 = pink-warm	Cold + hypo + hypoxia trio	Rewarm slow + glucose
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★ GRADE — ACT

Grade	Act
Cold stress	Warm room + cap + skin + feed
Moderate	Radiant + KMC + glucose + monitor
Severe <32	NICU + slow rewarm + O2 + lines
Prevent	10 links + no bath 24 h + transport warm

⚠ FATAL TRAP

Hot-water bags + heater-blast + rapid rewarm + 'warm enough' burns + crashes! Servo + skin-probe + slow + glucose + O2 + sepsis hunt (cold = infection flag!) + document curve.

★ NORCET HIT

HIT: Weigh + plot + feed + pee + danger card at discharge. Winter + night + power-cut + transport = cold peaks — plan warmth for each!

★ PNC PEARL: Warmth is a vital sign in neonates.

► Cold + lethargic + glucose low. Trio?

Rewarm slow + glucose + sepsis/O2 watch.

Rate + recessions + grunt + flaring + O2 need = score it! RDS (preterm + ground-glass) vs TTN (term-CS + wet) vs MAS (meconium + patchy) vs PPHN (labile + echo) — CPAP + surfactant + antibiotics + NICU ladder!

Score: rate + grunt + O2	RDS vs TTN vs MAS vs PPHN	CPAP + surfactant + NICU
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Σ CAUSE — CLUE — ACT

Cause	Clue	Act
RDS	Preterm + grunt + ground-glass	CPAP + surfactant + caffeine
TTN	Term-CS + wet + fast	O2 + watch + feeds slow
MAS	Meconium + patchy + PPHN	Suction-history + vent + echo
PPHN	Labile + echo shunt	iNO? + minimal handling + ICU

⚠ FATAL TRAP

100% O2 blast + vigorous bagging + 'wait till morning' + cold transport = ROP + air-leak + death! Blended + CPAP + NPO + glucose + lines + X-ray + gases + senior + transport ventilated.

★ NORCET HIT

HIT: Silverman-Anderson scores work + response. Surfactant early-rescue beats late! Pneumothorax (asymmetric + shift) = needle + drain + calm.

★ PNC PEARL: Grunt + rate + O2 — score, don't stare.

► Preterm + grunt + ground-glass. Ladder?

CPAP + surfactant + caffeine + NICU + gases.

<24 h + palms/soles + rapid rise + preterm/sick = DANGER (exchange zone)! Hour-chart (Bhutani) + lights + fluids + stop? (continue feeds!) + Coombs/G6PD/sepsis hunt — kernicterus (arch + high cry) never forgiven!

<24 h/palms =
danger

Chart + lights
+ feeds on

Arch + high cry
= kernicterus!

◆ TYPE — ACT

Type	Act
Physiological	Day 2–3 + well + feeds + watch + chart
Pathological	<24 h/palms/rapid/sick = lights + workup
Lights	Eyes covered + temp + fluids + feeds + chart
Exchange	Threshold + sick + encephalopathy + NICU

⚠ FATAL TRAP

Sunlight-only + stopping feeds + 'yellow is normal for a week' + late charting kernicteruses babies!
Hour-specific chart + covered eyes + temp + hydration + cause (hemolysis/sepsis/G6PD/hypothyroid) + follow-up hearing.

★ NORCET HIT

HIT: ABO/Rh + DCT + G6PD + sepsis + thyroid + biliary (pale stools + dark urine >2 wks = URGENT scan)! Stop breast? NEVER — feed through lights!

★ PNC PEARL: Clock the yellow, chart the hour.

► 18-h yellow + palms + lethargic. Zone + act?

Danger — lights + workup + exchange-ready + NICU.

NEONATAL CONVULSIONS + TETANUS

Jittery stops on hold; SEIZURE doesn't + eyes + apnea! Glucose + calcium + sepsis + HIE + bleed + metabolic hunt + ABCs + loader + NICU — tetanus (stiff + lockjaw + spasms, unclean cord) = ICU + immunoglobulin + silence!

Hold stops jitter;
seizure doesn't

Glucose + sepsis
+ HIE hunt

Tetanus: stiff +
spasms + cord

⚠ CAUSE — ACT

Cause	Act
Hypoglycemia	Dextrose + recheck + feeds plan
Sepsis/meningitis	Cultures + drugs + LP + HDU
HIE/bleed	Cooling + USG + loader + NICU
Tetanus	ICU + immunoglobulin + quiet + wound

⚠ FATAL TRAP

Phenobarbitone-first without glucose + LP + sepsis cover + 'evil eye' oil + loud handling = missed cause + status! ABCs + glucose + loader + cause-ladder + EEG + follow-up (CP/epilepsy risk).

★ NORCET HIT

HIT: Subtle (cycling + apnea) commonest in preterms! Pyridoxine + metabolic (ammonia/lactate) + IEM hunt in refractory. Cord + maternal Td history in tetanus!

★ PNC PEARL: Sugar first, loader second, cause always.

► Jittery + stops on hold + glucose low. Act?

Feed/dextrose + recheck + watch — jitter, not seizure.

Head-to-toe + anus + palate + hips + eyes + heart + cord at EVERY birth! Caput crosses, cephalohematoma doesn't; clavicle heals, plexus needs physio — screen + refer + counsel + surgery link + card!

Anus+palate+hips
+eyes day one

Caput crosses;
cephalo doesn't

Screen + refer
+ card

▲ FIND — ACT

Find	Act
Cleft/anus/hips/heart	NPO plan + echo + surgery link + card
Caput/cephalo/clavicle	Watch + jaundice + sling + physio
Plexus/facial	Physio + eye care + documents + follow-up
Multiple	Syndrome + karyotype + counsel + support

⚠ FATAL TRAP

Forceful arm-pull + 'massage the lump' + missed hips/anus + no card = palsy + late DDH + obstruction + lost follow-up! Gentle + Barlow/Ortolani + red-reflex + pulses + anus + counsel + dated return.

★ NORCET HIT

HIT: Femoral pulses (coarctation) + murmur + cyanosis = CCHD screen + echo + PGE where duct-dependent! Talipes + spina + hydrocephalus = folate + surgery + shunt plan.

★ PNC PEARL: Undress fully, miss nothing, card everything.

► Absent red reflex + white pupil. Suspect + refer?

Cataract/retinoblastoma — eye emergency + oncology link.

Exposed ≠ infected — syrup + EID + cotrimoxazole + safe feed till 18-mo antibody! Rh baby (DCT+, jaundice + anemia + hydrops history) = lights + top-up/exchange + IVIG + Hb track + development watch!

Exposed ≠ infected	Syrup + EID + cotrimoxazole	Rh: lights + top-up/exchange
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◆ BABY — PLAN

Baby	Plan
HIV-exposed	NVP syrup + EID 6 wks/6/12 mo + 18-mo antibody
Cotrimoxazole	From 6 wks till negative + feeds safe
Rh/HDN	DCT + Hb + lights/exchange + IVIG + track
Both	Growth + neuro + vaccines + card + follow-up

⚠ FATAL TRAP

Mixed feeding + missed EID + 'no test till 18 months' + shared/sold syrup = transmission + resistance + death! DOT + demo + ASHA + bring-back + partner ART + disclosure counsel.

★ NORCET HIT

HIT: EID DNA-PCR beats antibody <18 mo. Exchange + top-up thresholds + bilirubin/albumin + neuro (BIND) guide Rh care. BCG timing per stability!

★ PNC PEARL: Syrup + test + safe feed — till eighteen months says free.

► Exposed + 6 wks + well. Trio?

EID + cotrimoxazole + safe-feed counsel + card.

FLUIDS + FEEDS (EBM/TUBE/FORMULA)

EBM first + tube when weak + formula only indicated! Volume + calories + sterile + slow + watch (distension + residuals + NEC!) — cup/paladai beats bottle (confusion + infection)!

EBM first always	Tube when weak + slow	Cup beats bottle!
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Σ FEED — RULE

Feed	Rule
EBM	Express + store + label + warm + cup
Tube	Size + length + aspirate + slow + elevation
Formula	Indication + sterile + ratio + cup + counsel
Watch	Distension + residuals + stools + weight

⚠ FATAL TRAP

Bottle + over-dilute/concentrated + fast push + 'top-up till full' = NEC + sepsis + aspiration! Sterile + measured + slow + burp + KMC + handwash + demo + card + HBNC follow-up.

★ NORCET HIT

HIT: Trophic + advance per protocol + fortifier where indicated. IV fluids = calculated + labeled + pump + sugar + line care + review daily!

★ PNC PEARL: Slow + sterile + mother's own wins.

► Preterm + distension + residuals. Act?

Hold + aspirate + NPO + X-ray + NEC watch + senior.

SIDS, BEREAVEMENT + RECORDS/DRUGS

Back-to-sleep + empty crib + no smoke + room-share = SIDS down! Loss = name + hold + rituals + milk care + autopsy offer + grief + next-plan + staff debrief. Records + drug safety close every loop!

Back-sleep +
empty crib

Loss: hold +
rituals + plan

Records + drug
safety loop

★ ITEM — RULE

Item	Rule
SIDS-safe	Back + firm + empty + room-share + no smoke
Bereavement	Hold + name + rituals + milk + autopsy + follow-up
Records	Birth + NICU + shots + screens + card + MCTS
Drugs	Dose/weight + dilution + double-check + allergy

⚠ FATAL TRAP

Front-sleep + soft bedding + smoke + 'forget + next' after loss + verbal milligrams = death + trauma + error! Safe-sleep demo + grief + preconception + error-report + learn — dignity always.

★ NORCET HIT

HIT: Aminophylline/cafeine + antibiotics + vitamin K + surfactant + prostaglandin = weight-dose + pump + monitor + levels where needed!

★ PNC PEARL: Safe sleep tonight, held memory forever.

► Loss + mother wailing + 'take it away'. First 4?

Hold/offer + name + rituals + grief + milk + plan.

GYNAE HISTORY + EXAMINATION + BSE

Ask LMP + cycles + discharge + pain + lumps + bladder/bowel + sexual + smears + parity + surgery — chaperone + drape + consent ALWAYS! BSE: monthly self-check + CBE + mammo ladder (lump + dimpling + nodes = urgent triple test)!

LMP + cycles +
lump + smears

Chaperone +
drape always

Lump + nodes =
triple test!

★ STEP — DO

Step	Do
History	LMP, cycles, discharge, pain, lumps, bladder, smears
Exam	Chaperone + drape + vitals + breast + abdomen + pelvic
BSE	Monthly + mirror + palpate + axilla + report change
Triple	Exam + imaging + biopsy for every lump

⚠ FATAL TRAP

Pelvic exam without chaperone/consent + mocking discharge complaints + 'lump is nothing' without triple test = assault + missed cancer! Document + refer <2 wks for red-flag lumps/bleeds.

★ NORCET HIT

HIT: Red flags: postmenopausal bleed, postcoital bleed, breast dimpling, fixed hard lump, rapid fibroid growth. Pap + VIA + HPV per age!

★ PNC PEARL: Every lump earns three tests.

► Postmenopausal spotting + bulky uterus. Trio?

Endometrial cancer hunt — USG + biopsy + urgent referral.

CONGENITAL ANOMALIES + AMENORRHEA

Mullerian defects (septate/bicornuate/agenesis) + imperforate hymen (cyclic pain, no flow!) + Turner (webbed + streak) — primary amenorrhea (no menses by 15 + breasts / 13 no puberty) = outflow vs ovary vs brain ladder!

No flow + pain
= blocked

No menses 15 +
= workup

Turner: webbed
+ streak

▲ CAUSE — CLUE — ACT

Cause	Clue	Act
Imperforate hymen	Cyclic pain + bulge + no flow	Cruciate + drain + counsel
Mullerian agenesis	No uterus + normal ovaries	Vaginal + psych + fertility plan
Turner	Short + webbed + streak ovaries	GH + estrogen + cardiac/renal
Secondary stop	Pregnancy FIRST + TSH/prolactin + PCOS	

⚠ FATAL TRAP

D&C;/hormones-without-test for primary amenorrhea + missed pregnancy test in secondary = disaster!
Pregnancy test FIRST every stop + USG + hormones + karyotype ladder + counsel.

★ NORCET HIT

HIT: Asherman (post-D&C; scanty + infertility) = hysteroscopy. PCOS (oligo + acne + hirsutism + PCO) = lifestyle + metformin + ovulation plan!

★ PNC PEARL: Pregnancy test before every amenorrhea theory.

► 15-yr cyclic pain + no flow + bulge. Act?

Imperforate hymen — USG + cruciate + counsel.

Primary (ovulatory teens, prostaglandin cramps, NSAID + heat wins) vs **SECONDARY** (endometriosis/fibroid/PID — investigate!!) PMS/PMDD (luteal mood + breast + bloat, SSRI + lifestyle + diary) — red-fever/bleed/heavy = not 'just cramps'!

**Primary: NSAID
+ heat wins**

**Secondary =
investigate**

**Diary + SSRI
for PMDD**

◆ TYPE — ACT

Type	Act
Primary	NSAID + heat + exercise + reassure + school plan
Secondary	USG + infection/endometriosis hunt + treat cause
PMS/PMDD	Diary + lifestyle + SSRI + counsel + safety
Red with pain	Fever/bleed/mass = emergency lane

⚠ FATAL TRAP

'Period pain is fate' + NSAID-only for secondary + missed PID/ectopic/endometriosis = infertility + rupture + chronic pain! Red + new + severe + unresponsive = scan + senior.

★ NORCET HIT

HIT: Endometriosis triad (dysmenorrhea + dyspareunia + infertility) + chocolate cysts. School/work leave + hotline + pain diary empower!

★ PNC PEARL: New, severe, unresponsive — scan it.

► Teen + ovulatory cramps + NSAID relief. Type + plan?

Primary — NSAID + heat + exercise + reassure.

ABNORMAL UTERINE BLEEDING (PALM-COEIN)

PALM (Polyp, Adenomyosis, Leiomyoma, Malignancy) + COEIN (Coag, Ovulatory, Endometrial, Iatrogenic, Not classified)! Acute flood + shock = resuscitate + TXA + hormones/balloon/OT; chronic = Hb + cause + medical vs surgical ladder!

**PALM = structural
scan finds**

**COEIN = functional
labs find**

**Flood + shock =
resuscitate!**

Σ LETTER — MEANS — ACT

Letter	Means	Act
P/A/L	Polyp/adeno/fibroid	USG + hysteroscopy + remove
M	Malignancy (postmeno!)	Biopsy + stage + oncology
C/O	Coag/ovulatory	Clotting + hormones + TXA
E/I/N	Endometrial/iatrogenic	LNG-IUS + review drugs

⚠ FATAL TRAP

Hysterectomy-first for every heavy period + missed postmenopausal cancer + anticoag-ignorant D&C; = harm! Step: TXA + hormones + LNG-IUS + ablation + myomectomy before hysterectomy + histology always.

★ NORCET HIT

HIT: Postmenopausal ANY bleed = cancer till proven (biopsy <2 wks)! IUD + anticoag + thyroid + von Willebrand screens in teens!

★ PNC PEARL: PALM you see, COEIN you test.

► Postmenopausal bleed + thick lining. Act?

Biopsy + staging + urgent oncology referral.

Ascending killers: discharge + pain + fever + cervical motion tenderness = PID! Broad antibiotics + partner + abstinence + IUD review — Fitz-Hugh (liver sticks) + TOA + infertility stalk delays!

CMT + fever = PID	Partner + abstinence	TOA/infertility = delay cost
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★ BUG — ACT

Bug	Act
Gono/chlamydia	Ceftriaxone + doxy/azithro + partner + condoms
Trich/BV/candida	Treat + pH + preterm/pregnancy watch
TOA/severe	Admit + IV + USG + drainage/surgery
Prevent	Screen + IUD strings + safe + vaccine (HPV/HBV)

⚠ FATAL TRAP

Single-dose her + 'inform him later' + sex-during-treatment = ping-pong + TOA + tubes lost!
Both treated + abstinence till cure + retest + condom demo + documented + dated.

★ NORCET HIT

HIT: CMT + adnexal + fever = PID even without discharge! IUCD with PID = remove after antibiotics started. Infertility + ectopic = PID's late bills!

★ PNC PEARL: Tubes scar silently — treat loudly, both sides.

► CMT + fever + discharge + adnexal pain. Act trio?

PID — broad antibiotics + partner + abstinence.

Mackenrodt + levator fail (multiparity + age + chronic cough/constipation!) = down/out! Stage + pessary + Kegels + surgery (hysterectomy + repair + sling) + cough/constipation fix + FP — decubitus ulcer + retention = urgent!

Multiparity + age + strain = down	Pessary + Kegels + fix cause	Ulcer/retention = urgent!
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▲ TYPE — CLUE — ACT

Type	Clue	Act
Cystocele	Bladder bulge + stress leak	Pessary + Kegels + sling
Rectocele	Rectal bulge + stool digitation	Fiber + repair + stool plan
Uterine	Cervix descending + backache	Ring + hysterectomy + vault fix
Urgent	Ulcer/bleed/retention	Admit + pack + OT + catheter

⚠ FATAL TRAP

Pushing back + 'live with it' + missed ulcer/cancer on prolapse + no FP = strangulation + malignancy + pregnancy-on-prolapse! Examine + Pap + Hb + surgery fitness + vault + sling + review.

★ NORCET HIT

HIT: POP-Q stages. Kegel + weight + cough-treat + constipation-fix prevent + preserve. Post-op: no lift/strain/sex 6 wks + vault + bladder + bowel check!

★ PNC PEARL: Lift the cause, support the floor.

► Prolapse + ulcer + retention. Lane?

Urgent — admit + pack + catheter + OT plan.

**3 Ds: dysmenorrhea + dyspareunia + DIS (infertility) + chocolate cysts + fixed tender uterus!
Suppress (OCP/progestin/GnRH) + conserve fertility + excise + pain + psych + ART bridge —
chronic, team-managed, believed!**

**3 Ds + chocolate
= endo**

**Suppress +
conserve + excise**

**Chronic + team
+ believed**

◆ SITE — ACT

Site	Act
Ovarian (chocolate)	USG + cystectomy + fertility + ART link
Deep (bowel/bladder)	Excision + multidisciplinary + stoma/urology
Adenomyosis	Bulky + heavy + LNG-IUS + hysterectomy if done
Pain/psyche	Analgesia + hormones + support + work plan

⚠ FATAL TRAP

'Just bad periods' + repeated laps without plan + pregnancy-promise as cure = years lost + fertility gone!
Early USG + referral + fertility preservation + pain + psych + registry + review.

★ NORCET HIT

HIT: CA-125 rises (non-specific!). Malignant change rare + counsel. Pregnancy + menopause pause (not cure) — document + dated review!

★ PNC PEARL: Believe pain, map lesions, guard fertility.

► Triad + chocolate cyst + infertility. Plan?

Excise + fertility + ART link + pain + review.

FIBROID (LEIOMYOMA)

Estrogen-fed + reproductive-age + heavy/prolonged + pressure + infertility + red degeneration in pregnancy! Myomectomy saves uterus, UAE/ablation selected, hysterectomy when done — sarcoma-rapid growth = oncology!

**Heavy + pressure
+ infertility**

**Myomectomy saves
uterus**

**Rapid + postmeno
= oncology!**

Σ TYPE — ACT

Type	Act
Submucosal	Bleed + infertility — hysteroscopic remove
Intramural	Bulk + pressure — myomectomy vs UAE
Subserosal	Torsion/pressure — select + watch
Red degeneration	Pregnancy pain + fever — rest + analgesia

⚠ FATAL TRAP

Hysterectomy-first for every fibroid + missed sarcoma + myomectomy-without-blood/consent = harm! Fibroid map + Hb + fertility wish + options + histology + rupture-risk counsel (future pregnancy)!

★ NORCET HIT

HIT: GnRH shrinks pre-op. UAE spares uterus but pregnancy counsel! Post-myomectomy = CS next (rupture risk) + blood + review!

★ PNC PEARL: Map, wish, options — then knife.

► Rapid postmenopausal fibroid growth. Act?

Sarcoma hunt — staging + urgent oncology.

Bleed-between + infertility + postmenopausal spot = POLYP till proven (hysteroscopy + histology)! Ovarian cyst: functional vs dermoid vs endometrioma vs malignant (ROMA/CA-125 + solid + ascites + postmeno = oncology)! Bartholin: word catheter + marsupialization, NOT squeeze!

**Between-bleed =
polyp hunt**

**Solid + ascites +
postmeno = malig**

**Bartholin: word
not squeeze!**

▲ FIND — ACT

Find	Act
Endometrial/cervical polyp	Hysteroscopy + remove + histology
Functional cyst	Watch + rescan + OCP + pain plan
Dermoid/endometrioma	Excise + fertility + spill-care
Bartholin abscess	Word + marsupialization + antibiotics

⚠ FATAL TRAP

Squeezing Bartholin + 'functional always' + missed torsion/malignancy = sepsis + lost ovary + late cancer!
Torsion (sudden + vomit) = Doppler + OT detorse/fix;
RMI + senior + oncology for suspicious!

★ NORCET HIT

HIT: Torsion twists + vomits + tender + Doppler-flow lost — minutes matter, ovary-saving detorsion! Vulval cyst + ulcer + itch in elderly = biopsy!

★ PNC PEARL: Between-bleeds biopsy, sudden pain operates.

► Sudden pelvic pain + vomit + tender mass. Act?

Torsion — Doppler + emergency OT + ovary-save.

OVARIAN TORSION + GENITOURINARY FISTULA

Twist + vomit + tender + flow-lost = DETORSE NOW (ovary-saving, Doppler + OT + fix)! Fistula (continuous leak post-obstructed/CS) = dye + catheter + centre + repair + dignity + FP — both clock-sensitive, both counsel-heavy!

**Twist + vomit
= detorse**

**Leak continuous
= fistula**

**Minutes +
dignity both**

⚠ EVENT — ACT

Event	Act
Torsion	Doppler + OT detorse + fix + cyst plan
Ruptured cyst + shock	Blood + OT + products + ICU
VVF/RVF	Dye + catheter + centre + repair
Post	FP + psych + review + next plan

⚠ FATAL TRAP

Oophorectomy-first for every torsion + pads-forever for fistula + no FP = ovary + dignity lost! Detorse + fix + observe + histology; fistula timing + expert + tissue + catheter + reintegration.

★ NORCET HIT

HIT: Pregnancy + ART + large cyst = torsion-prone — pain + vomit + tender = Doppler STAT! Fistula prevention = partograph + no prolonged obstruct!

★ PNC PEARL: Untwist fast, link leaks early.

► Torsion + viable ovary on Doppler. Save how?

Detorse + fix + cyst plan + observe + review.

Fibroadenoma (young, mobile, rubbery) vs cyst vs abscess vs CANCER (hard + fixed + dimpling + nodes + discharge)! Triple test EVERY lump + mammogram + receptor + stage + surgery + chemo/radio + hormones + arm + psych!

Mobile rubbery = fibroadenoma	Hard + fixed + nodes = cancer	Triple test every lump!
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◆ FIND — ACT

Find	Act
Fibroadenoma	Triple + excise + reassure + review
Abscess/mastitis	Antibiotics + drain + feed plan
Cancer lump	Stage + receptors + surgery + adjuvant
Arm/psyche	No BP/needle + sleeve + support + prosthesis

⚠ FATAL TRAP

'Fibroadenoma' without triple + BP/needle on operated arm + no receptor test = missed cancer + lymphedema + wrong drugs! Age + family + BRCA + screen + MDT + survivorship + FP (pregnancy after)!

★ NORCET HIT

HIT: Mammogram + US + core + ER/PR/HER2 + nodes + mets workup. Lymphedema arm-care lifelong + sleeve + physio + weight!

★ PNC PEARL: Receptors rule therapy — test before treat.

► Hard fixed + nodes + dimpling. Ladder?

Triple + stage + MDT + receptors + plan.

Postcoital bleed + foul + backache + cachexia = CERVIX till proven! Pap + VIA + HPV + colposcopy + biopsy + stage + surgery/radio-chemo + palliative + HPV vaccine 9–14 (2 doses) + screen-and-treat camps!

Postcoital bleed = cervix	VIA + HPV + colposcopy	Vaccine 9–14 + camps!
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Σ STEP – DO

Step	Do
Screen	VIA + HPV + Pap per age + camps
Confirm	Colposcopy + biopsy + stage + MDT
Treat	Surgery/RT-chemo + bladder/bowel + palliative
Prevent	HPV 9–14 + condoms + tobacco-stop + FP

⚠ FATAL TRAP

Bleed + 'infection' antibiotics-months + no speculum + missed vaccine window = stage-IV + death! Speculum EVERY bleed + biopsy + stage + tobacco + partner + psych + hospice link.

★ NORCET HIT

HIT: VIA + cryo/thermal single-visit! Staging + fistula + ureter + pain + bleed + discharge + family + will + legacy in palliative!

★ PNC PEARL: Screen today, vaccine tomorrow's mothers.

► Postcoital bleed + foul + growth seen. Act?

Biopsy + stage + urgent oncology + support.

ENDOMETRIAL, OVARIAN + VULVAL CANCERS

Postmenopausal bleed = ENDOMETRIAL till biopsy! Bloating + early satiety + ascites + CA-125 = OVARIAN (debulking + chemo)! Vulval itch + ulcer in elderly = biopsy! Stage + MDT + palliative from day one!

Postmeno bleed
= biopsy

Bloat + ascites
= ovary

Vulval ulcer
elderly = cut!

▲ CANCER — ACT

Cancer	Act
Endometrial	TVS + biopsy + stage + surgery ± RT
Ovarian	CA-125 + CT + debulk + chemo + genes
Vulval/vaginal	Biopsy + wide + nodes + RT + wound
All	Pain + psych + hospice + family + will

⚠ FATAL TRAP

D&C-without-staging; + 'ovarian cyst'
watch-with-ascites + vulval steroid-months = upstaged
+ lost! RMI + senior + oncology + frozen-section +
debulk + chemo + genes + review.

★ NORCET HIT

HIT: Lynch + BRCA families = screen + prophylaxis
counsel! Bloating + urinary + pelvic pain + satiety =
ovary symptom index + USG!

★ PNC PEARL: Bleed, bloat, ulcer — biopsy all three.

► Bloating + ascites + CA-125 high. Ladder?

CT + debulk + chemo + genes + MDT + review.

Puberty (8–13 breasts, 15 menses) + precocious/delayed ladders; MENOPAUSE (12-mo stop + flashes + bone + heart + mood) + HRT/MHT (lowest + shortest + yearly + clot/cancer screen) + calcium + exercise + quit-smoke!

Puberty 8–13 ladders	Menopause 12-mo stop + care	HRT lowest + yearly review
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★ STAGE — CARE

Stage	Care
Precocious/delayed	Bone age + hormones + MRI + counsel
Menopause	Flashes + sleep + mood + bone + heart plan
HRT/MHT	Lowest + shortest + yearly + screen
Bone/heart	Calcium + D + exercise + quit + lipids

⚠ FATAL TRAP

HRT-forever + no screen + 'menopause needs nothing' + missed bleed-cancer = clot + cancer + fracture!
Indication + window + yearly + bleed-report + mammo + lipids + bone + stop-plan.

★ NORCET HIT

HIT: STRAW + FSH + AMH stage + counsel. Vaginal estrogen (local) + lubes + pelvic floor + sex counsel + psych + support groups!

★ PNC PEARL: Lowest, shortest, yearly — HRT's three locks.

► HRT + new postmeno bleed. Act?

Stop + biopsy + review + oncology if needed.

12 months (<35) / 6 months (≥35) trying = INFERTILITY! Male (40%) + ovulatory + tubal + uterine + unexplained — BOTH partners together from visit ONE + semen FIRST (easy!) + weight + tobacco + age honesty!

12 mo / 6 mo rule	Both + semen FIRST	Age + weight + tobacco truth
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★ CAUSE — SHARE — CLUE

Cause	Share	Clue
Male	~40%	Count/motility/morph + varicocele + heat
Ovulatory	~25%	PCOS + thyroid + weight + prolactin
Tubal	~20%	PID + TB + surgery + HSG
Uterine/unexplained	Rest	Fibroid/septum/endo + normal tests

⚠ FATAL TRAP

Woman-only testing + 'try harder' years + quack guarantees + debt = blame + bankruptcy + bef! Couple + semen-first + timed + weight + tobacco + age + budget + stop-line counsel.

★ NORCET HIT

HIT: TB endometritis + PID + surgery = tubal top in India! Semen: abstinence 2–7 d + lab + repeat + urology. Age cliff 35 + steep 40!

★ PNC PEARL: Semen first, blame never.

► 2-yr trying + normal cycles + no male test. First?

Semen analysis + couple counsel + weight/tobacco.

History + exam + semen + ovulation + tubes + uterus + reserve — ORDERED, not random! Day-3 FSH/LH + AMH + USG + HSG + semen + laparoscopy-selective + TB workup + karyotype-recurrent — staged + costed + consented!

Ordered ladder not random	Semen + AMH + HSG core	Stage + cost + consent
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▲ TEST — RULE

Test	Rule
Semen (2×)	Abstinence + lab + urology + lifestyle
Ovulation + reserve	Day-21 progesterone + AMH + AFC + TSH/prolactin
Tubes + uterus	HSG + USG + hysteroscopy/lap selective
Special	TB + karyotype + antisperm + DFI select

⚠ FATAL TRAP

HSG-in-infection + laparoscopy-first + 'package tests' debt + no consent = harm! Screen + treat + stage + cheapest-first + couple + budget + stop-line + support group.

★ NORCET HIT

HIT: HSG day 6–12 + antibiotics + allergy + pregnancy-excluded! AMH + AFC + age predict (not guarantee). TB PCR + histology + ATT + wait!

★ PNC PEARL: Cheapest answer first, costliest last.

► HSG due + vaginal discharge + fever. Act?

Defer + treat infection + reschedule + consent.

Grief + guilt + pressure + money + myths — counsel BOTH + families + timelines + budgets + stop-lines + adoption + childfree dignity! Every visit: hope + honesty + privacy + no-blame + support group!

**Both + family
+ budget**

**Hope + honesty
+ stop-line**

**Adoption + dignity
always open**

◆ NEED — GIVE

Need	Give
Grief/guilt	Listen + normalize + psych + group
Money/time	Cost + success + stop-line + pause rights
Myths/pressure	Facts + family meet + privacy + FP
Options	Treat + ART + donor + adoption + childfree
⚠ FATAL TRAP Guarantee + sex-selection + debt + 'defective wife' + no stop-line = exploitation + ruin! Ethical + legal + documented + second-opinion + cooling-off + complaint channel + law.	★ NORCET HIT HIT: Success + multiple + OHSS + ectopic + preterm counsel BEFORE ART. Donor + surrogacy + adoption = parenthood, not failure — dignity!

★ PNC PEARL: Parenthood has many doors — open all.

► Demanding guarantee + selling land for IVF. Counsel?

Honest odds + budget + stop-line + options + pause.

Clomid/letrozole + scan + trigger + timed/IUI + luteal = first rungs! ICSI (one sperm + egg) for male/failed-fertilization — OHSS + multiples + ectopic counsel + freeze-all option + budget + stop-line!

Clomid/letrozole
+ scan + trigger

ICSI = one sperm
+ egg

OHSS + multiples
counsel!

Σ STEP – DO

Step	Do
OI	Day 2–5 + scan + trigger + timed + luteal
IUI	Washed + catheter + rest + test date
ICSI	Male/failed + embryo + transfer + freeze
Watch	OHSS + multiples + ectopic + budget

⚠ FATAL TRAP

Gonadotropins-without-scan + 'more follicles better' + no OHSS plan = hyperstim + ICU + death! Monitoring + freeze-all + cabergoline + fluids + admission thresholds + consent.

★ NORCET HIT

HIT: Letrozole-first in PCOS + metformin + weight!
Luteal progesterone + test + USG + ectopic watch + multiples reduction counsel where legal!

★ PNC PEARL: Monitored follicles, not maximal ones.

► OI + 8 follicles + pain + vomit. Fear + act?

OHSS — fluids + labs + freeze-all + admit watch.

Down-regulate + stimulate + trigger + retrieve + fertilize + culture + transfer + luteal + test! OHSS + multiples + ectopic + preterm + budget + single-embryo counsel + freeze + consent + registry — lab + love + law!

Stimulate +
retrieve + grow

Transfer + luteal
+ test date

Single embryo
+ freeze!

★ STAGE — CARE

Stage	Care
Stimulate	Dose + scan + trigger + OHSS watch
Retrieve + lab	Sedation + fertilize + grade + freeze
Transfer	Single + catheter + rest + luteal
Test	hCG date + USG + multiples + ectopic watch

⚠ FATAL TRAP

Three-embryo transfer + no OHSS plan + 'sure twins' + debt + no consent = preterm + loss + ruin! Single + freeze + budget + stop-line + psych + support + law + registry.

★ NORCET HIT

HIT: Success age-graded + cycle-wise + honest! Donor eggs + surrogacy + adoption discussed BEFORE despair. Lab + air + culture + witness + label discipline!

★ PNC PEARL: One good embryo beats three gambles.

► Transfer + OHSS + pain + vomit + scanty urine. Act?

Admit + fluids + labs + freeze-all + monitor.

SURROGACY, DONATION, CRYO + ADOPTION

Altruistic + registered + consented + insured + legal! Donor (sperm/ovum/embryo) + cryo + adoption (CARA) = many doors — exploitation + commercial + coercion = crime + report + protect!

**Altruistic +
registered only**

**Donor + cryo
+ consent**

**CARA adoption
+ home study**

▲ DOOR — RULE

Door	Rule
Surrogacy	Altruistic + clinic + insurance + consent + court
Donation	Screened + anonymous + consent + records
Cryo	Consent + duration + fate + fees + law
Adoption	CARA + home study + court + follow-up

⚠ FATAL TRAP

Commercial deal + coercion + no insurance + 'private adoption' cash = trafficking + jail! Legal + registered + documented + counseled + cooling-off + complaint + protect surrogate/child.

★ NORCET HIT

HIT: ART law + surrogacy law + CARA + PCPNDT intersect — board + records + audit + consent + age/marriage/infertility criteria + registry!

★ PNC PEARL: Legal doors only — the rest are traps.

► Cash-for-womb offer + unregistered clinic. Act?

Refuse + report + legal counsel + protect + document.

Believe + private + examine + treat + collect + link + follow! Injuries + EC/PEP + STI + MLC + sealed samples + shelter/181 + psych + court + dignity — joint with abuser NEVER, consent + chaperone ALWAYS!

**Believe + private
+ chaperone**

**EC/PEP + MLC
+ sealed**

**Shelter + 181
+ follow-up**

⚠ STEP — DO

Step	Do
Listen/examine	Private + consent + body-map + photos
Treat	Injuries + EC/PEP + STI + Td + pregnancy test
Collect	Sealed + labeled + chain + police + court
Link	Shelter + legal + psych + 181 + review

⚠ FATAL TRAP

Bath + wash + delay + joint + 'compromise' + no samples = evidence + justice lost! No bath + same clothes + sealed + timely + believed + linked + followed — nurse = first protector.

★ NORCET HIT

HIT: One-stop Sakhi + child (POCSO) + disability + trafficking + conflict + custody + compensation + rehab + education + livelihood chain!

★ PNC PEARL: First belief heals more than first medicine.

► Assault + torn clothes + police untold. Trio?

Treat + preserve + MO/police + shelter + follow.

GYNAE PROCEDURES + DRUGS OVERVIEW

Pap + biopsy + hysteroscopy + D&C; + MVA + sterilization + myomectomy + hysterectomy + chemo/radio + hormones + antibiotics + TXA + iron + vaccines — indication + consent + count + sample + handover, every time!

Indication + consent + list	Count + sample + label	Handover + review all
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◆ ITEM — RULE

Item	Rule
Screen/biopsy	Pap/VIA + colposcopy + histology + stage
Uterine	Hysteroscopy/D&C;/MVA + Rh + FP + histology
Surgery	Myomectomy/hysterectomy + count + VTE + review
Drugs	Hormones + TXA + iron + antibiotics + allergy

⚠ FATAL TRAP

Unlabeled histology + uncounted pack + missed pregnancy test + no FP = lost cancer + sepsis + suit!
Label bedside + count aloud + test + FP + card + dated + debrief + audit.

★ NORCET HIT

HIT: ERAS + VTE + wound + bladder + bowel + sex + HRT + bone + psych + support + survivorship + registry close every loop!

★ PNC PEARL: Label, count, counsel — procedure's trinity.

► Post-D&C; + no void + distended. Act?

Retention — encourage + catheter + measure + cause.

GYNAE EMERGENCY RED-FLAG ENGINE

Bleed + pain + shock + fever + mass + discharge + collapse = ENGINE ON! Ectopic + torsion + TOA + bleed + shock + assault + post-op sepsis — ABCs + pregnancy test + USG + blood + OT + senior + senior + document!

**Bleed/pain/shock
= engine on**

**Pregnancy test
+ USG first**

**Blood + OT +
senior now!**

Σ FLAG — LANE

Flag	Lane
Bleed + shock	Resuscitate + TXA + blood + OT + count
Pain + mass + vomit	Torsion/TOA/ectopic + Doppler + OT
Fever + foul + tender	Sepsis + cultures + IV + source
Collapse/assault	ABCs + MLC + shelter + ICU + follow

⚠ FATAL TRAP

'Gynae OPD tomorrow' + vitals-skipped + pregnancy-untested + USG-delayed + blood-unready = dead + ruptured + septic! Red = resuscitation bay + bundle + roles + kit + clock + handover.

★ NORCET HIT

HIT: MEOWS + shock index + lactate + urine + Hb + coag + crossmatch + consent + family + money + transport + feedback = chain unbroken!

★ PNC PEARL: Red flags fly patients, not files.

► Shock + bleed + positive UPT + tender mass. Lane?

Ectopic rupture — blood + OT + consent + ICU.